

EVERGREEN COMMUNITY CARE INC.

OCCUPATIONAL HEALTH AND SAFETY PROGRAM

Comprehensive Binder — Residential Care Services

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Approved By	Executive Director and Board of Directors
Governing Legislation	Workers Compensation Act (BC); OHS Regulation (BC Reg. 296/97); WorkSafeBC Policies and Guidelines; BC Human Rights Code; Residential Care Regulation; FOIPPA
Applies To	All employees, contractors, students, and volunteers at all Evergreen Community Care worksites
Total Sections	20 Sections + 10 Appendices

DOCUMENT INFORMATION

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Applies To	All employees (full-time, part-time, casual, fixed-term), contractors, practicum students, and volunteers at all Evergreen Community Care residential facilities and offices
Related Documents	HR Policy Binder (ECC-HR-POL-2026-001); Residential Care Policy Manual (ECC-RC-POL-2026-002); Emergency Response Procedures; Vehicle Safety Policy

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1. HEALTH AND SAFETY POLICY STATEMENT

Regulatory Authority: Workers Compensation Act s.115–116; OHS Regulation Part 1

1.1 Statement of Commitment

Evergreen Community Care Inc. is unconditionally committed to providing and maintaining a safe and healthy work environment for every worker — including employees, contractors, students, and volunteers — at all worksites. We believe all workplace injuries and occupational illnesses are preventable. Health and safety is a core organizational value, not a compliance exercise.

This commitment extends to the safety of the children and youth in our care. A safe, stable workplace for staff is the foundation of safe, stable residential care.

Leadership Commitment

Health and safety receives the same level of leadership attention, resource allocation, and accountability as any other organizational priority. No production or operational target justifies compromising worker safety.

1.2 Our Commitments to Every Worker

- We will:
- Comply fully with the Workers Compensation Act, OHS Regulation, and all WorkSafeBC requirements applicable to our operations
- Provide all information, instruction, training, and supervision necessary for every worker to perform their work safely
- Proactively identify and control workplace hazards before they cause harm, using the hierarchy of controls
- Investigate all incidents and near-misses promptly and thoroughly to prevent recurrence
- Meaningfully consult with workers through the Joint Health and Safety Committee on all matters affecting health and safety
- Never tolerate retaliation, penalization, or adverse treatment of any worker who reports a hazard, refuses unsafe work, or participates in the OHS program in good faith
- Provide a physically and psychologically safe workplace free from violence, bullying, harassment, and discrimination
- Integrate health and safety into all levels of planning, hiring, purchasing, contracting, and daily operations

1.3 Shared Responsibility

Health and safety is everyone's responsibility. The Board provides governance and resources. Management provides systems, training, and supervision. Supervisors enforce safe practices and respond to hazards. Workers follow safe work procedures, use protective equipment, and report all hazards. The JHSC monitors performance and advocates for improvement. This Program describes how each of these responsibilities is exercised.

Responsibility	Party Accountable
Organizational governance, resource allocation	Board of Directors
Overall OHS performance and program accountability	Executive Director

Program implementation at facility level	Program Managers / Supervisors
Day-to-day coordination, records, WorkSafeBC liaison	Health and Safety Coordinator
Safe daily work practices, hazard reporting	All Workers
Monitoring, consultation, recommendations	Joint Health and Safety Committee

Signed: _____ Date: _____

Executive Director, Evergreen Community Care Inc.

2. ROLES AND RESPONSIBILITIES

Regulatory Authority: Workers Compensation Act s.115–117; OHS Regulation s.3.1–3.3

2.1 Board of Directors

The Board of Directors holds governance responsibility for the OHS Program:

- Approves the OHS Program, associated policies, and annual OHS budget
- Receives and reviews the annual OHS performance report from the Executive Director
- Ensures the organization has adequate resources, staffing, and systems to fulfil OHS obligations
- Maintains organizational accountability for health and safety outcomes at the governance level

2.2 Executive Director

The Executive Director holds ultimate operational accountability for OHS performance:

- Champions visible safety leadership and models the safety culture expected of all staff
- Allocates adequate budget, staff time, and physical resources to health and safety
- Reviews all serious incidents, JHSC recommendations, and WorkSafeBC orders
- Ensures corrective actions are completed and systemic issues addressed at the organizational level
- Signs the annual OHS performance report presented to the Board

2.3 Health and Safety Coordinator

The Health and Safety Coordinator manages day-to-day OHS program administration:

- Maintains all OHS records, statistics, and documentation in compliance with retention requirements
- Acts as the primary organizational liaison with WorkSafeBC
- Supports and coordinates the JHSC, including scheduling, agenda preparation, and tracking action items
- Coordinates OHS training scheduling, tracks compliance, and reports training gaps to supervisors
- Conducts or coordinates the annual comprehensive OHS program review
- Maintains the OHS bulletin board content at each facility (current, accurate, and WorkSafeBC-compliant)

2.4 Program Managers and Facility Supervisors

Program Managers and Facility Supervisors implement the OHS Program at the worksite level:

- Ensure every new worker receives site-specific safety orientation before beginning unsupervised work
- Conduct or delegate monthly workplace inspections and document deficiencies and corrective actions
- Investigate incidents and near-misses occurring under their supervision and submit reports within required timelines
- Identify, correct, and document workplace hazards within their authority; escalate hazards beyond their authority immediately
- Ensure PPE, safe work equipment, and relevant safe work procedures are available, accessible, and followed
- Enforce compliance with safe work procedures consistently and without discrimination
- Support workers through the return-to-work process following injury or illness

- Respond promptly and without retaliation to all work refusals filed under Section 10

2.5 All Workers

Every worker — employee, contractor, student, or volunteer — has the following rights and responsibilities:

Responsibility	Detail	Regulation
Work safely	Follow all safe work procedures and instructions; do not engage in horseplay or any unsafe act	OHS Reg. s.3.11
Use PPE	Wear and properly use all required personal protective equipment for assigned tasks	OHS Reg. s.8.3
Report hazards	Report all hazards, near-misses, and incidents to a supervisor immediately; no hazard is too minor to report	WCA s.116(2)
Refuse unsafe work	Exercise the right to refuse work that poses an unusual or undue hazard, per Section 10	WCA s.3.12
Participate in training	Complete all required OHS training within mandated timelines	OHS Reg. s.3.23
Cooperate in investigations	Participate honestly and fully in incident and near-miss investigations	WCA s.172
Protect others	Take reasonable precautions to protect the health and safety of coworkers and youth in care	WCA s.116(1)

Worker Rights

Every worker has the right to: know about workplace hazards; participate in health and safety decisions; refuse unsafe work; and report concerns to WorkSafeBC — all without fear of retaliation. These rights cannot be waived, restricted, or disciplined away.

3. JOINT HEALTH AND SAFETY COMMITTEE (JHSC)

Regulatory Authority: Workers Compensation Act s.125–135; OHS Regulation Part 3

3.1 Legal Requirement

The Workers Compensation Act requires workplaces regularly employing 20 or more workers to establish and maintain a Joint Health and Safety Committee. Evergreen Community Care maintains an organization-wide JHSC covering all facilities. Facilities with fewer than 20 workers have a designated Worker Health and Safety Representative who fulfils equivalent functions at the site level and is integrated into the organization-wide JHSC structure.

3.2 Composition

Composition Element	Requirement
Minimum total members	Four (4) members; more for larger workforces
Worker representation	At least half of all members must represent workers (non-management)
Worker representatives	Selected by the workers they represent, or by their union where applicable
Employer representatives	Appointed by management
Co-chairs	Two co-chairs: one worker-selected, one management-appointed
Term of office	Two (2) years, renewable; staggered to maintain continuity
Posting requirement	Committee membership list posted on OHS bulletin board at each facility
Vacancy	Filled within 30 days; functions continue with remaining members

3.3 Mandate and Statutory Functions

The JHSC has the following statutory functions under the Workers Compensation Act and OHS Regulation:

- Identify workplace hazards and unsafe conditions and recommend corrective actions to management
- Participate in and review the results of workplace inspections, accident investigations, and hazard assessments
- Receive and investigate worker health and safety complaints and concerns; respond in writing within 21 days
- Participate in work refusal investigations as required under WCA s.3.12
- Participate in the annual review of the OHS Program and recommend improvements
- Consult on the development, implementation, and review of OHS policies and safe work procedures
- Promote health and safety education and awareness among workers
- Advise on training requirements and monitor OHS training compliance

3.4 Meetings

Requirement	Standard
Minimum frequency	Monthly (more frequently following any serious incident)
Quorum	At least one worker representative and one management representative
Minutes	Recorded at every meeting, posted for worker access within 7 days,

	retained minimum 2 years
Action item tracking	Open corrective actions tracked meeting-to-meeting until resolved and closed
Annual reports	JHSC submits annual summary to the Executive Director and Board
Emergency meetings	Called within 48 hours of any fatality, critical injury, or WorkSafeBC stop-work order

3.5 JHSC Member Rights

- Paid time to attend meetings, conduct inspections, investigate work refusals, and participate in training
- Access to all OHS records, inspection reports, incident reports, and investigation results
- Right to make written recommendations to management; management must respond in writing within 21 days
- Protection from reprisal for carrying out JHSC functions in good faith

3.6 JHSC Training Requirements

Training	Audience	Timeline
JHSC basic certification (8 hours, WorkSafeBC-recognized)	All new JHSC members	Within 6 months of joining the committee
Workplace inspection training	JHSC members conducting inspections	Before conducting first independent inspection
Incident investigation training	JHSC members participating in investigations	Within 6 months of joining the committee
Annual OHS update	All JHSC members	Annually; WorkSafeBC online resources or equivalent

4. HAZARD IDENTIFICATION AND RISK ASSESSMENT

Regulatory Authority: OHS Regulation s.4.1–4.3; WorkSafeBC Hazard Assessment Guidelines

4.1 Purpose and Obligation

Evergreen Community Care is required by the OHS Regulation to identify workplace hazards, assess the risk those hazards pose, and implement controls to eliminate or minimize the risk. Hazard identification is an ongoing, proactive process — not a one-time event.

4.2 Hazard Identification Methods

Method	When Used
Monthly workplace inspections	Scheduled; carried out by JHSC and supervisor — see Section 20
Pre-task / job hazard analysis	Before any new, modified, or unfamiliar task begins
Worker hazard reports	Ongoing; any worker may submit a hazard report at any time
Incident and near-miss investigations	After every incident or near-miss — see Section 11
New equipment or facility risk assessment	Before introducing new equipment, chemicals, or facility changes
Annual comprehensive hazard review	During the annual OHS program review
JHSC recommendations	Following meetings, inspections, or investigations
WorkSafeBC inspection or order	Following any regulatory visit or order

4.3 Workplace Hazard Categories — Residential Care

The following hazard categories have been identified as relevant to Evergreen Community Care's residential care operations. Each is addressed in detail in Sections 5–7 and the applicable safe work procedures:

Hazard Category	Examples Specific to Residential Care
Workplace violence	Physical assault by youth; aggression from family members; threatening visitors
Psychological / traumatic stress	Secondary trauma; compassion fatigue; critical incident exposure; shift work isolation
Ergonomic / musculoskeletal	Lifting youth; restraint-related strain; prolonged standing; poor workstation setup
Biological	Blood, bodily fluids, airborne pathogens (respiratory illness, TB, MRSA); sharps exposure
Chemical / WHMIS	Cleaning products; disinfectants; medications; cooking chemicals; pest control agents
Slips, trips, and falls	Wet floors; outdoor environments; stairs; carrying items while moving
Fire and evacuation	Cooking fires; youth-set fires; evacuation of youth with mobility or behavioural challenges
Vehicle and transportation	Youth transport; staff driving to and from activities; adverse weather

Working alone or in isolation	Overnight shifts; isolated areas; single-staff coverage
Electrical	Faulty equipment; youth tampering with outlets; extension cord misuse
Noise	Residential environment; distress behaviours; alarm systems
Temperature extremes	Outdoor activities; inadequate heating or cooling at facility

4.4 Risk Assessment Matrix

Once identified, hazards are assessed using the following Likelihood × Severity matrix to prioritize control efforts:

Likelihood / Severity	Negligible	Minor	Moderate	Major	Catastrophic
Rare	Low	Low	Low	Medium	High
Unlikely	Low	Low	Low	Medium	High
Possible	Low	Low	Medium	High	High
Likely	Low	Low	Medium	High	Critical
Almost Certain	Low	Low	Medium	High	Critical

Risk Level	Required Response
Low	Monitor; address through routine maintenance or standard operating procedures
Medium	Documented corrective action plan within 30 days; interim controls in place
High	Corrective action within 7 days; interim controls implemented immediately on identification
Critical	Immediate action required; work in the affected area stopped until hazard is eliminated or controlled to an acceptable level

4.5 Hierarchy of Controls

When selecting controls for identified hazards, Evergreen Community Care follows the Hierarchy of Controls in order from most to least effective. Multiple controls are often combined. PPE alone is never considered an acceptable sole control where higher-level controls are reasonably practicable:

Control Level	Definition Examples in Residential Care
1. Elimination (most effective)	Physically remove the hazard from the workplace Remove broken furniture; discontinue a chemical product
2. Substitution	Replace the hazard with something less dangerous Use a safer cleaning product; switch to needleless medication delivery
3. Engineering Controls	Physically isolate workers from the hazard Install door alarms; improve lighting; install safety locks
4. Administrative Controls	Change how work is done Safe work procedures; staffing ratios; shift scheduling; training
5. PPE (least effective alone)	Protect the individual worker as the last line of defence Gloves, masks, eye protection, cut-resistant gloves

4.6 Hazard Assessment Records

Every formal hazard assessment is documented on the Hazard Assessment and Control Form (Appendix E), retained in the OHS records system for a minimum of 3 years for active hazards, and reviewed at least annually or whenever conditions change. The JHSC receives copies of all new and updated hazard assessments.

5. SAFE WORK PROCEDURES — GENERAL

Regulatory Authority: OHS Regulation s.4.3; WorkSafeBC Safe Work Practices Guidelines

5.1 Purpose and Development

Safe Work Procedures (SWPs) document the step-by-step, safe way to complete tasks that present significant hazards. Evergreen Community Care develops SWPs collaboratively with workers and the JHSC. SWPs are reviewed annually and immediately following any incident involving the covered task.

5.2 Slip, Trip, and Fall Prevention

- Floors are inspected at the start of each shift and after any spill; wet floors are cleaned immediately or cordoned off with a wet floor sign until dry
- Stairways are kept clear of all objects at all times; handrails are used when carrying items on stairs
- Outdoor walkways, steps, and ramps are checked for ice or wet conditions; sand or salt is applied immediately where required
- Non-slip footwear is required for staff working in kitchen areas or completing outdoor tasks in wet conditions
- Ladders are used for any task requiring a worker to climb more than one step; chairs and boxes are not used as platforms

5.3 Safe Lifting and Material Handling

- Workers assess the weight and shape of any load before lifting; loads estimated over 23 kg (50 lbs) require mechanical assistance or a two-person lift
- Safe lifting technique: feet shoulder-width apart; bend at hips and knees; keep back straight; hold load close to body; avoid twisting while carrying
- Team lifts are coordinated by a designated leader who gives clear direction: 'Ready — lift — step — set down'
- Mechanical aids (hand trucks, carts, dollies) are available and used for heavy or awkward loads where possible

5.4 Electrical Safety

- Only functioning electrical equipment with intact cords and plugs is used; damaged equipment is tagged out and removed from service immediately
- Extension cords are used only temporarily and are not routed under carpets, through doorways, or across walkways
- Outlets are not overloaded; multi-outlet adaptors that do not include a circuit breaker are prohibited
- Electrical panels are kept clear of obstructions with a minimum 1-metre clearance at all times
- Workers do not perform any electrical work beyond replacing fuses or resetting breakers; all other electrical work is done by a licensed electrician

5.5 Kitchen and Food Preparation Safety

- Knives are used only for their intended purpose; cut-resistant gloves are available and used for high-risk cutting tasks
- Stove burners and oven are checked at the end of each use and before each shift; the stove is not left unattended while in use

- A Class K fire extinguisher is mounted within reach of the cooking area; all staff are trained in its use
- Kitchen grease filters above the stove are cleaned monthly; the cleaning date is logged
- Hot liquids are carried with two hands and transported using a lidded container where possible; youth are warned before hot items are moved through shared areas
- Food safety temperatures are maintained: hot food above 60°C; cold food at or below 4°C; frozen food at or below -18°C

5.6 Housekeeping and Sanitation

- Work and living areas are maintained in clean, orderly condition at all times; clutter that creates trip hazards or limits evacuation routes is removed immediately
- Waste is disposed of promptly in appropriate containers; sharps waste (needles, lancets) is placed in a certified sharps container — never in regular waste
- High-touch surfaces (door handles, light switches, shared equipment) are disinfected at minimum daily and after any illness event in the home
- All cleaning products are stored in their original labeled containers in a locked cupboard; products are never mixed

5.7 Outdoor Safety

- Outdoor areas used by youth are inspected before each use for hazards: broken equipment, glass, needles, animal waste, standing water, or ice
- Outdoor inspection deficiencies are reported to the supervisor and documented; areas with serious hazards are restricted until corrected
- Lawn mowing, shoveling, and exterior maintenance tasks are assessed for heat, cold, or fatigue risk; workers are provided adequate breaks, water, and appropriate PPE
- Sun protection (sunscreen, hats, shade breaks) is encouraged for extended outdoor activities; extreme heat work procedures apply when the temperature exceeds 35°C

5.8 Locking, Tagging, and Energy Control (Lockout/Tagout)

Where energy isolation is required during maintenance or repair of powered equipment:

- The equipment is shut down, de-energized, and isolated from all energy sources before maintenance begins
- A lockout tag is affixed clearly indicating the lock owner, date, and reason; only the lock owner may remove it
- After lockout, the equipment is tested to confirm it is de-energized before any person contacts it
- Maintenance requiring lockout/tagout is performed only by staff who have received specific lockout training; youth are not present in the work area during such maintenance

6. SAFE WORK PROCEDURES — RESIDENTIAL CARE SPECIFIC

Regulatory Authority: OHS Regulation Part 4, 5, 6; WCA s.115; Residential Care Regulation s.7

6.1 De-escalation and Behaviour Management Safety

Working with children and youth who have complex trauma histories and behaviour support needs requires trained, consistent de-escalation skills as the primary approach to safety. Physical response is always a last resort.

- All direct-care staff complete Non-Violent Crisis Intervention (NVCi) or equivalent training before working independently with youth, and renew annually
- De-escalation is attempted as the first response to any escalating situation: create space; use a calm voice and non-threatening posture; offer choices; reduce environmental stimulation
- Staff are never required to physically manage a behavioural crisis alone; backup is called before escalation to physical intervention
- Any physical intervention used must be within the approved methods covered in NVCi training; unapproved restraint methods are prohibited
- All physical interventions are documented in the shift log and on the Incident Report form within 2 hours, regardless of outcome

6.2 Medication Handling Safety

- Medications are handled only by staff with current Medication Administration authorization on file
- All medications are stored in a locked cabinet; controlled substances in a double-locked storage and counted at every shift change
- Liquid medications are dispensed using calibrated measuring devices; oral syringes are used where indicated
- Staff wash hands before and after every medication administration; gloves are worn when handling topical or compounded medications
- Sharps (lancets, insulin syringes) are placed immediately in a certified sharps container; sharps containers are sealed and replaced when three-quarters full
- Medication disposal follows BC Medications Return program requirements; no medications are flushed or placed in general waste

6.3 Infection Prevention and Control

Residential care environments carry elevated risk of infectious disease transmission. All staff follow standard precautions regardless of the known or perceived infection status of a youth.

- Standard Precautions: assume all blood and bodily fluids are potentially infectious; use appropriate barriers for all contact
- Handwashing: wash hands with soap and water for minimum 20 seconds before and after each youth interaction, after removing gloves, before and after food handling, and after bathroom use
- Gloves: worn for all contact with blood, bodily fluids, mucous membranes, non-intact skin, and contaminated items
- Mask and eye protection: worn where splashing of blood or bodily fluids is possible (first aid, personal care with active bleeding)
- Contaminated surfaces: cleaned with an approved disinfectant immediately; blood spills cleaned with bleach solution (1:10 dilution) or approved equivalent
- Ill youth: youth with communicable illness are provided care in their room where possible; staff caring for ill youth wear appropriate PPE per the specific illness guidance

- Ill staff: staff with active fever, vomiting, diarrhea, or diagnosed communicable illness must not report to work until symptom-free per public health guidance

6.4 Blood and Bodily Fluid Exposure Response

An exposure to blood or bodily fluids is a serious workplace incident requiring immediate and documented response:

- Step 1: Wash the exposed area immediately with soap and water (skin exposure) or flush with water (eye or mucous membrane exposure) for minimum 15 minutes
- Step 2: Notify the supervisor immediately
- Step 3: Seek medical assessment within 2 hours for assessment of post-exposure prophylaxis (PEP) risk, regardless of perceived risk level
- Step 4: Complete an Incident Report form and a First Aid Record entry within 24 hours
- Step 5: Report to WorkSafeBC if the exposure results in injury or required medical attention beyond first aid

6.5 Safe Sleeping and Nighttime Monitoring

- Overnight staff conduct a minimum wellness check of each youth every 30 minutes or as specified in the youth's Individual Service Plan or Behaviour Support Plan
- Wellness checks are documented in the shift log with time and observation; skipped checks are reported to the supervisor immediately
- Staff remain on-site and alert throughout the overnight shift; sleeping on duty while youth require monitoring is prohibited
- Any youth not in bed or in an unexpected location during wellness checks triggers the Missing Youth Protocol (HR Policy Binder Section 11G) immediately

6.6 Personal Care and Hygiene Assistance

- When assisting youth with personal care (bathing, dressing), gloves are worn; gown and mask are used if there is risk of bodily fluid contact
- Personal care is provided with the youth's informed agreement; any resistance or distress is respected and the approach adjusted; supervisors are consulted where needed
- Two staff are present for personal care where the youth's behaviour history or physical support needs create safety risks
- After personal care, all soiled materials are disposed of per biohazard or general waste procedures as appropriate; the work area is disinfected

6.7 Animal Safety

Where therapeutic animals, pets, or service animals are present in a facility or program:

- All animals are vaccinated, licensed, and have a current veterinary health certificate on file
- Staff and youth interactions with animals are supervised; no animal is left alone with youth unsupervised
- Animal bites or scratches are washed immediately, reported to the supervisor, and assessed medically if the skin is broken
- Staff with animal allergies are identified at orientation; facilities notify HR if an animal is introduced so accommodation can be assessed

7. WORKPLACE VIOLENCE PREVENTION PROGRAM

Regulatory Authority: OHS Regulation s.4.24–4.31; WCA s.115(2)(d); WorkSafeBC Policy D3-115-2

This section constitutes Evergreen Community Care's written Workplace Violence Prevention Program as required under OHS Regulation s.4.27. Residential care is classified as an inherently higher-risk environment for workplace violence due to the nature of the population served.

7.1 Definition of Workplace Violence

Workplace violence means the attempted or actual exercise by a person, other than a worker, of any physical force so as to cause injury to a worker, and includes any threatening statement or behaviour that gives a worker reasonable cause to believe that the worker is at risk of injury. This includes:

- Physical assault: hitting, kicking, biting, scratching, spitting, throwing objects at staff
- Threats: verbal, written, or physical gestures that reasonably cause fear of harm
- Psychological violence: intimidation, stalking, harassment that causes fear for safety
- Property destruction that implicitly threatens personal safety

7.2 Annual Violence Risk Assessment

The Program Manager, in consultation with the JHSC, conducts a written violence risk assessment at each facility at minimum annually, using the Workplace Violence Risk Assessment Template (Appendix F). The assessment addresses:

Risk Factor	Residential Care Considerations
Client / youth population	Trauma history, known aggression behaviours, current mental health status, identified triggers
Physical environment	Sightlines, isolated areas, layout barriers, lack of escape routes, access control weaknesses
Staffing levels and patterns	Single-worker coverage, overnight isolation, shift change vulnerability, supervisor availability
History of incidents	Violence incidents in past 12 months by location, time, youth, and type
High-risk situations	Bedtime routines, medication refusals, family visits, community outings, awakenings
External visitor risk	Former residents, family members with conflict history, unknown callers

7.3 Prevention Measures

Based on the current risk assessment, the following prevention controls are implemented:

- Individual Behaviour Support Plans (BSPs): maintained for all youth with a documented history of physical aggression; reviewed at each ISP review period and following any violence incident
- Staffing ratios: maintained at levels that allow safe de-escalation without staff isolation; minimum two-staff coverage during identified high-risk periods
- Communication systems: all on-duty staff have access to a communication device (phone, walkie-talkie, or personal alarm) throughout their shift
- Environmental design: annual review of facility layout to minimize blind spots, isolated areas, and dead ends; corrective actions tracked through the JHSC
- Access control: exterior doors are locked; all visitors are verified and escorted; former residents without authorization are not admitted

- Mandatory NVCI: all direct-care staff trained in Non-Violent Crisis Intervention before independent work; refreshed annually

7.4 Reporting All Violence Incidents

All violent incidents are reported and documented — no incident is too minor to report:

- All physical assaults, regardless of whether the worker was injured, are reported to the supervisor immediately and documented on the Incident Report form within 2 hours
- All threats (verbal or written) that cause a worker to fear for their safety are documented in the shift log and on the Incident Report form within 24 hours
- The JHSC reviews all violence-related incident reports at monthly meetings and recommends additional preventive measures
- WorkSafeBC is notified for any incident that results in worker injury, time lost from work, or required medical attention beyond basic first aid

7.5 Post-Incident Support

Following a violent incident, Evergreen Community Care provides:

- Immediate removal from duty of the affected worker for a safety check and brief defusing with the supervisor
- Medical attention arranged immediately if there is any injury, regardless of perceived severity
- Formal Critical Incident Stress Debrief (CISD) within 48–72 hours for all staff involved in significant violent incidents
- Access to EFAP trauma-focused counselling at no cost to the worker
- Modified duties or temporary schedule adjustment where the worker needs time to recover before returning to high-demand shifts

7.6 No Tolerance for Retaliation

No worker will face reprisal, penalization, or adverse employment action for: reporting a violence incident; refusing work based on a credible violence threat; or participating in a violence-related investigation. Retaliation is itself a serious violation of the Workers Compensation Act and this policy.

8. OCCUPATIONAL STRESS AND PSYCHOLOGICAL SAFETY

Regulatory Authority: OHS Regulation s.4.24; WCA s.5.1; WorkSafeBC Psychological Health and Safety Guidelines

8.1 Policy Statement

Evergreen Community Care recognizes that psychological health and safety is as important as physical safety. Residential care work involves regular exposure to acute and chronic stressors including: vicarious trauma from working with children who have experienced abuse and neglect; secondary traumatic stress from critical incidents; compassion fatigue from sustained empathic engagement; and the challenges of shift work, isolation, and complex interpersonal dynamics.

The organization is committed to proactively identifying and addressing psychosocial hazards, not merely responding to crisis.

8.2 Psychosocial Hazard Identification

The following psychosocial hazards are recognized in Evergreen Community Care's residential care environment:

Psychosocial Hazard	Description and Risk in Residential Care
Secondary traumatic stress	Cumulative impact of regular exposure to trauma disclosures, abuse histories, and crisis events of youth in care
Compassion fatigue	Diminished capacity for empathy resulting from sustained emotional demands of caregiving work
Critical incident trauma	Acute stress reactions following major incidents (assaults, youth suicidal crisis, death, major emergencies)
Shift work and fatigue	Disrupted sleep, isolation during overnight shifts, fatigue-related judgment impairment
Role conflict and ambiguity	Unclear expectations between professional care and personal connection; limit-setting tension
Organizational factors	Workload imbalance, insufficient staffing, limited control over scheduling, inadequate support
Workplace bullying and harassment	Conduct by coworkers or management that causes humiliation or intimidation — see Section 7 of HR Binder
Racial trauma and microaggressions	Disproportionate impact on racialized and Indigenous staff; cumulative burden of discrimination

8.3 Prevention and Control Measures

- Reflective supervision: individual reflective supervision sessions between direct-care staff and their Program Manager, minimum monthly, focused on emotional wellbeing and practice reflection
- Team debriefs: brief team debrief built into every team meeting agenda to normalize discussion of stressors and emotional load
- EFAP access: confidential Employee and Family Assistance Program available to all workers and family members from day one; active promotion at orientation and following any critical incident
- Critical Incident Stress Management: structured CISD offered within 48–72 hours of any significant critical incident (see HR Binder Section 9E)

- Workload monitoring: Program Managers monitor workloads and staffing adequacy; workers are encouraged to raise workload concerns through supervision or the JHSC without fear of negative response
- Organizational culture: leadership models and reinforces a culture in which vulnerability, asking for help, and taking care of oneself are valued and respected, not penalized

8.4 WorkSafeBC Mental Disorder Claims

A worker who develops a mental disorder as a result of a work-related traumatic event, a pattern of events, or a significant work stressor may be entitled to WorkSafeBC mental disorder compensation under WCA s.5.1. Evergreen Community Care:

- Does not discourage workers from filing legitimate WorkSafeBC mental disorder claims
- Assists workers in accessing the WorkSafeBC claims process and provides requested documentation within required timelines
- Treats all mental disorder claims with the same respect and seriousness as physical injury claims

9. IMPAIRMENT AND FITNESS FOR DUTY

Regulatory Authority: OHS Regulation s.4.20; WCA s.115–116; BC Human Rights Code s.13

9.1 Purpose

The safety of children and youth in Evergreen Community Care's residential programs requires every worker to be fully fit for duty throughout their shift. This section governs impairment from all sources including alcohol, cannabis, controlled substances, prescription and over-the-counter medication, extreme fatigue, and acute mental health crisis.

9.2 Fitness for Duty Standard

Every worker must report to and remain fit for duty for the entirety of every scheduled shift. A worker is impaired when, for any reason, their cognitive or physical capacity to safely perform their assigned duties is reduced below what is required for safe care delivery.

9.3 Prohibited Conduct

Prohibited Conduct	Detail
Reporting to work impaired	Applies to any substance or condition that impairs judgment, reaction time, coordination, or cognitive function
Alcohol — pre-shift window	No alcohol within 8 hours before a shift; 12 hours before any shift involving vehicle operation
Cannabis — pre-shift window	No cannabis within 12 hours before any shift; cannabis impairment cannot be assessed by smell alone — behaviour and functioning are the standard
Controlled substances	No use of non-prescribed controlled substances before or during any shift
On-site possession	Illicit substances may not be on organizational property or in organizational vehicles at any time
On-site consumption	No alcohol, cannabis, or controlled substances may be consumed on organizational property or during shifts, including breaks taken off-site

9.4 Prescription and Over-the-Counter Medication

Workers taking prescription or over-the-counter medication that may impair alertness, reaction time, motor coordination, or cognitive judgment are required to inform their supervisor before the start of any shift in which they will take such medication. The worker is not required to name the medication; confirmation from a pharmacist that it may impair work performance is sufficient. Supervisors treat this disclosure confidentially. Modified duties may be assigned based on functional limitations.

9.5 Reasonable Grounds Assessment

Where a supervisor has reasonable grounds to believe a worker may be impaired while on duty (observable indicators include: slurred speech, unsteady gait, impaired coordination, disorientation, unusual odour, dramatic mood or behaviour change, or direct disclosure), the supervisor:

- Immediately removes the worker from direct client contact
- Conducts a private, respectful conversation with the worker
- Arranges safe transportation home if the worker cannot safely operate a vehicle
- Documents the observations and actions taken
- Reports to the Program Manager and Human Resources
- Does not conduct physical testing (breathalyzer, drug screen) without legal advice confirming entitlement

9.6 Fatigue as an Impairment Risk

Fatigue in residential care is a recognized OHS hazard. Workers are responsible for arriving at work adequately rested. Supervisors monitor for fatigue, particularly in workers who have worked extended hours or consecutive shifts. A fatigued worker must not be assigned to high-acuity or high-demand situations without adequate rest. See also: Overtime and Lieu Time Policy (HR Binder s.5A); Staff Scheduling Policy (HR Binder s.11P).

9.7 Duty to Accommodate Substance Dependency

Substance use disorder and addiction are disabilities under the BC Human Rights Code. An employee who voluntarily discloses a dependency and actively engages in treatment will not be subject to progressive discipline for the disclosure alone. Accommodation may include: EFAP referral; medical leave; graduated return-to-work. Active impairment during a shift remains a safety issue regardless of dependency status.

10. RIGHT TO REFUSE UNSAFE WORK

Regulatory Authority: Workers Compensation Act s.3.12; OHS Regulation s.3.12

Fundamental Worker Right

Every worker in British Columbia has the legal right to refuse work they have reasonable cause to believe presents an undue hazard to their health and safety or to the health and safety of another person. This right cannot be waived, restricted, contracted away, or disciplined.

10.1 When the Right to Refuse Applies

A worker may refuse to perform a task or work at a location where they have reasonable cause to believe that the work or workplace presents an undue hazard. Reasonable cause is assessed from the perspective of a reasonable worker in that situation — the hazard does not have to be certain, only reasonably believed.

10.2 Step-by-Step Refusal Process

Step	Action Required	Responsible Party
1 — Stop and notify	Stop work immediately and notify the supervisor of the specific unsafe condition	Worker
2 — Remain available	Stay at or near the worksite in a safe location; do not leave the workplace	Worker
3 — Supervisor investigation	Supervisor investigates the reported unsafe condition with the worker and, where available, a JHSC member	Supervisor + JHSC
4a — Hazard confirmed	If the hazard is confirmed, interim control measures are implemented immediately; work resumes only when it is safe	Supervisor
4b — Hazard not confirmed	If the supervisor concludes no undue hazard exists, they explain their conclusion to the worker in detail	Supervisor
5 — Continued refusal	Worker may continue to refuse if not satisfied with the supervisor's investigation; a JHSC member or WorkSafeBC officer is requested	Worker + JHSC
6 — WorkSafeBC	If refusal continues, WorkSafeBC is contacted; a prevention officer investigates and makes a determination	WorkSafeBC
7 — Documentation	All steps, conversations, findings, and resolutions are documented on the Work Refusal Documentation Form (Appendix D)	Supervisor + Worker

10.3 Substitute Worker Restrictions

A substitute worker may perform the refused work only after being told, in the presence of a JHSC member or worker representative: that the work has been refused; the reason for the refusal; and that the refusal is under investigation. The substitute worker also has the right to refuse if they have reasonable cause to believe an undue hazard exists.

10.4 No Reprisal — Strict Prohibition

Evergreen Community Care strictly prohibits any form of reprisal, discipline, demotion, adverse schedule change, or other negative action against a worker for exercising or attempting to exercise the right to refuse unsafe work in good faith. Any supervisor or manager found to have retaliated against a worker for a safety refusal is subject to immediate disciplinary action, up to and including termination.

10.5 Documentation

Every work refusal is documented on the Work Refusal Documentation Form (Appendix D), regardless of how it is resolved. Completed forms are submitted to the Health and Safety Coordinator, reviewed by the JHSC, and retained for a minimum of 10 years.

11. INCIDENT REPORTING, INVESTIGATION, AND WORKSAFEBC REPORTING

Regulatory Authority: WCA s.172–176; OHS Regulation s.3.4; WorkSafeBC Reporting Requirements

11.1 What Must Be Reported Internally

Every worker must report the following to their supervisor immediately:

- Any work-related injury or illness, regardless of severity — including injuries that do not require medical attention beyond basic first aid
- Any near-miss event (an event that did not result in injury or damage but could have under slightly different circumstances)
- Any workplace hazard identified during work, not just during formal inspections
- Any violent incident or threat, including threats that did not result in physical contact
- Any blood or bodily fluid exposure

No Incidents Are Too Minor

Near-miss reporting is essential. Near-misses reveal hazards before they cause harm. Workers who report near-misses are contributing to organizational safety — not creating paperwork. No worker may be disciplined for reporting a hazard or incident in good faith.

11.2 Internal Incident Report Timeline

Incident Type	Internal Report Timeline
Any work-related injury or incident	Verbally to supervisor: immediately. Written Incident Report form: within 24 hours
Near-miss	Written Incident Report form: within 24 hours
Violent incident (no injury)	Written Incident Report form: within 2 hours
Violent incident (with injury)	Verbal to supervisor: immediately. Written Incident Report: within 2 hours
Exposure to blood or bodily fluid	Verbal to supervisor: immediately. Incident Report and First Aid Record: within 24 hours
Critical incident involving a youth (separate from worker safety)	Also report per Residential Care Policy Manual Critical Incident Reporting requirements

11.3 WorkSafeBC Mandatory Reporting

The following events must be reported to WorkSafeBC immediately (defined as as soon as possible — no later than the next business day for most events, immediately by phone for serious events):

Reportable Event	Reporting Method and Timeline
Serious injury: fracture of major bone, amputation, loss of consciousness, burn over 20% of body, eye injury with potential vision loss	Immediate telephone to WorkSafeBC (1-888-967-5377); followed by written report within 48 hours
Fatality at the workplace	Immediate telephone notification; scene is not disturbed without WorkSafeBC authorization
Major structural failure or	Immediate telephone notification; area secured

collapse	
Serious incident of workplace violence causing injury requiring medical attention beyond first aid	Written report (Form 7) within 48 hours of incident; immediate call if serious injury
Exposure to a harmful substance requiring medical attention	Written report within 48 hours
WorkSafeBC Employer's Report (Form 7)	Within 3 days of learning of any work-related injury requiring medical treatment beyond first aid
WorkSafeBC Worker's Report (Form 6A)	Worker completes within 1 year; organization encourages timely filing

11.4 Incident Investigation Requirements

All incidents and near-misses are investigated to identify contributing factors and prevent recurrence. The depth of investigation is scaled to the severity and learning potential of the event:

Investigation Level	Trigger
Basic investigation (supervisor-led)	First aid only, near-miss, property damage under \$5,000
Intermediate investigation	Injury requiring medical attention; violent incident; exposures
Full investigation	Lost-time injury; hospitalization; serious incident; repeat injury from same cause
Critical investigation	Fatality; critical injury; WorkSafeBC order; serious violence event

11.5 Investigation Process

- Preserve the scene: do not disturb the incident scene until the supervisor has assessed and documented it, except to prevent further injury or provide first aid
- Gather information: interview the injured worker (if able and willing), witnesses, and other involved parties separately; record statements
- Examine physical evidence: photograph the scene; document equipment, environmental conditions, and any contributing factors
- Identify root causes: go beyond the immediate cause (e.g., the worker slipped) to the underlying system or organizational factors (e.g., why was the floor wet and not addressed)
- Develop corrective actions: identify specific, measurable actions to prevent recurrence; assign each action to a named owner with a target completion date
- Share learnings: communicate incident learnings to relevant staff without identifying the affected worker unnecessarily; present JHSC with investigation results

11.6 First Aid Records

Every instance of first aid provided to a worker is recorded in the facility First Aid Record book (hard copy or electronic equivalent), with: date and time; worker name; nature of injury or illness; first aid treatment provided; name of first aid attendant; and follow-up (medical referral, WorkSafeBC claim). First Aid Records are retained for minimum 3 years.

12. EMERGENCY PREPAREDNESS AND RESPONSE

Regulatory Authority: OHS Regulation Part 4, s.4.13–4.14; BC Fire Code; Emergency Program Act

12.1 Emergency Response Plans

Evergreen Community Care maintains a written Emergency Response Plan for each facility, covering the emergency scenarios most likely to affect residential care operations. Plans are posted at each facility, reviewed annually, and updated following any emergency activation or significant change.

12.2 Fire Emergency

- On activation of any fire alarm: evacuate all occupants immediately using posted evacuation routes; do not use elevators; do not stop to gather personal belongings
- Before opening any door during evacuation, feel the door and frame for heat; if hot, use an alternate route
- Shift lead assigns evacuation roles before each shift: youth escort lead; room sweep staff; 911 caller
- All occupants assemble at the designated assembly point; head count completed and results reported to fire services immediately on arrival
- No one re-enters the building until fire services provide all-clear
- If a youth cannot be safely evacuated due to mobility or behavioural factors, move them to the nearest area of refuge, close doors between them and the fire, and advise fire services of their location immediately

12.3 Medical Emergency

- Call 911 immediately for: unconsciousness; difficulty breathing; suspected cardiac arrest or stroke; severe allergic reaction (anaphylaxis); significant injury including possible fracture, head trauma, or heavy bleeding; or any situation where the worker or youth's life may be at risk
- Begin first aid while 911 is called; stay on the line with the dispatcher for instructions
- Notify the supervisor immediately; the supervisor notifies the Program Manager
- Keep other youth away from the scene with support from a second staff member
- Complete an Incident Report form and First Aid Record within 24 hours; notify WorkSafeBC per Section 11 if it is a worker injury

12.4 Lockdown / Shelter-in-Place

A lockdown is initiated when there is an immediate threat to the safety of occupants from an external source (e.g., armed person in the area, police pursuit, directed threat to the facility):

- Lock all exterior doors immediately; do not open doors for anyone until directed by police or the Program Manager
- Move all youth and staff away from windows and doors to interior rooms or hallways
- Call 911 immediately if there is an active or imminent threat; follow police direction
- Do not attempt to investigate the threat or confront any person; remain quiet and sheltered in place
- Call the Program Manager and MCFD after-hours when it is safe to do so

12.5 Power Failure and Utility Loss

- Flashlights and battery-powered lanterns are kept in a designated, accessible location at each facility and checked monthly

- For power outages lasting more than 1 hour: notify the Program Manager; assess heating and food safety implications
- For extended outages: refrigerated medications are moved to a cooler with ice if temperature-sensitive; the prescriber is contacted for guidance
- For loss of heat lasting more than 2 hours in cold weather: youth are provided additional blankets and warm drinks; Program Manager determines whether temporary relocation is required
- For loss of running water: Program Manager is notified immediately; bottled water is made available; WorkSafeBC or health authority may require evacuation for extended outages

12.6 Severe Weather

- Severe weather warnings from Environment Canada are monitored; facility-specific severe weather plans are activated when applicable
- Community outings and transport are cancelled or modified during warnings for significant snowfall, ice, extreme wind, or extreme heat
- Extreme heat (temperature above 35°C or humidex above 40°C): windows and fans maximized; cool beverages available at all times; cool-down areas identified; youth and staff monitored for signs of heat illness; outdoor activities suspended

12.7 Emergency Drill Schedule

Drill Type	Frequency	Documentation
Fire evacuation (announced)	Quarterly at each facility	Drill record: date, time, participants, issues noted, corrective actions
Fire evacuation (unannounced, night/weekend)	Annually	Same as above; filed with JHSC
Lockdown / shelter-in-place	Annually	Tabletop or live drill; JHSC receives summary
Medical emergency tabletop	Annually (team meeting)	Meeting minutes
Missing youth protocol review	Annually or after any event	Meeting minutes; policy update if indicated
Utility failure response review	Annually	Team meeting or supervisor briefing

13. NEW AND YOUNG WORKER SAFETY ORIENTATION

Regulatory Authority: OHS Regulation s.3.22–3.23; WCA s.115(2)(b); WorkSafeBC Young and New Worker Program

13.1 Legal Requirements

The OHS Regulation requires employers to ensure that new workers receive basic OHS orientation before beginning work. This obligation applies to every new worker including employees, contractors, practicum students, and volunteers, regardless of prior experience or training.

Young workers (workers under 25 years of age) are at significantly elevated risk of workplace injury due to inexperience, reluctance to ask questions, and desire to demonstrate competence. Special attention is given to ensuring young workers are adequately oriented, supervised, and supported.

13.2 New Worker Orientation — Required Content

Every new worker receives the following orientation before beginning unsupervised work, documented on the New Worker Safety Orientation Checklist (Appendix A):

Orientation Topic	Details
OHS rights and responsibilities	Worker rights: to know, to participate, to refuse unsafe work; employer and worker responsibilities under the WCA
Organizational OHS Program overview	Location of OHS Program binder; JHSC contacts; how to report a hazard or incident
Site-specific hazard identification	Walk-through of the facility highlighting key hazards: violence risks, chemical storage, slips/trips, evacuation routes
Workplace violence prevention	Facility-specific violence risks; de-escalation approach; who to call for backup; how to report
Emergency procedures	Fire evacuation route and assembly point; medical emergency steps; lockdown procedure; location of first aid kit and extinguishers
Safe work procedures	Review of all SWPs relevant to the worker's assigned role before they begin those tasks
WHMIS orientation	SDS location; how to read a safety data sheet; what to do if exposed to a hazardous product
PPE	What PPE is required for the role; where it is stored; how to use and inspect it
Incident and hazard reporting	How to complete an Incident Report; who to report to; importance of near-miss reporting
Right to refuse unsafe work	Step-by-step process; assurance of no reprisal
Supervisor and JHSC contacts	Names, roles, and how to reach the supervisor and JHSC representative at the site

13.3 Orientation Timing and Sign-Off

- OHS orientation is completed before the worker performs any task not directly supervised by an experienced worker
- The orientation checklist is signed by both the worker and the orienting supervisor; the worker receives a copy
- The signed checklist is retained in the worker's personnel file and in the OHS records system
- For young workers, the supervisor conducts at minimum one formal check-in during the first week and again at the end of the first month to address any safety questions or concerns

13.4 Site-Specific Orientation for Existing Workers Assigned to New Locations

When an existing employee is assigned to work at a facility they have not previously worked at, the facility supervisor provides a site-specific orientation covering the unique hazards, evacuation routes, emergency contacts, and any site-specific safe work procedures before the worker begins work at that location.

14. CONTRACTOR, VISITOR, AND VOLUNTEER SAFETY

Regulatory Authority: OHS Regulation s.3.20–3.21; WCA s.118–119

14.1 Contractor Safety Obligations

Contractors who perform work at Evergreen Community Care facilities are responsible for complying with the Workers Compensation Act, OHS Regulation, and Evergreen Community Care's site-specific safety requirements while on site. As the prime contractor at each of its own facilities, Evergreen Community Care establishes the following requirements for all contractors:

- Before commencing work: provide proof of current WorkSafeBC registration and coverage; confirm their own workers are covered under their account
- Receive a site-specific safety orientation from the facility supervisor covering: emergency procedures, evacuation routes, hazard locations, visitor sign-in, and child safety requirements
- Report all incidents, injuries, and near-misses occurring on Evergreen Community Care's premises to the facility supervisor immediately
- Follow all posted safe work procedures and safety signage; comply with all directions from Evergreen Community Care supervisors on safety matters
- Never perform work alone in an area where children or youth are present without a staff member present
- Child safety: any contractor with direct or unsupervised access to children or youth must have a clear Vulnerable Sector CRC on file before commencing work

14.2 Prime Contractor Responsibilities

Where Evergreen Community Care directs the activities of two or more contractors on a single worksite, Evergreen Community Care is the prime contractor and must: establish an OHS program for the worksite; ensure each contractor knows their responsibilities; and coordinate health and safety activities between contractors to eliminate or minimize hazards arising from their combined activities.

14.3 Visitor Safety

- All visitors sign the visitor log on arrival; they are briefed on emergency evacuation procedures and assembly point by the staff member who greets them
- Visitors are escorted at all times; they do not enter hazardous work areas (utility rooms, kitchen during meal preparation, outdoor activity areas) without PPE appropriate to the area
- Visitors who witness or are involved in any incident on site are assisted to report it before leaving the facility

14.4 Volunteer and Practicum Student Safety

- Volunteers and practicum students complete the New Worker Safety Orientation (Section 13) before beginning any activities at Evergreen Community Care facilities
- Volunteers and students are assigned a designated supervisor who monitors their safety compliance and provides direction throughout their time at the facility
- Volunteers and students are not assigned tasks that carry significant hazard until they have demonstrated competency under supervision
- WorkSafeBC coverage for volunteers is confirmed through the Evergreen Community Care account if they are performing work that would normally be performed by a worker

15. PERSONAL PROTECTIVE EQUIPMENT (PPE) PROGRAM

Regulatory Authority: OHS Regulation Part 8; WorkSafeBC PPE Selection and Use Guidelines

15.1 PPE as Last Resort

Personal protective equipment is the last control in the hierarchy of controls. It is used where engineering and administrative controls alone are not sufficient to eliminate or adequately minimize the hazard. PPE does not eliminate the hazard — it reduces the worker's exposure to it. Evergreen Community Care provides, maintains, and replaces all required PPE at no cost to workers.

15.2 PPE Requirements by Task — Residential Care

Task / Hazard	Required PPE	Standard	Notes
Blood or bodily fluid contact	Nitrile examination gloves	ASTM D6319	Change between youth; dispose in appropriate waste container
Splashing risk (first aid with bleeding, personal care)	Gloves + face shield or goggles + gown / apron	ANSI Z87.1 (eye)	Required when splash to face, eyes, or clothing is possible
Medication handling (topical, compounded, cytotoxic)	Nitrile gloves; gown where indicated	As per medication SDS	Cytotoxic medications: follow pharmacist and prescriber guidance
Cleaning with bleach or strong disinfectants	Nitrile gloves; ventilation required; eye protection if splashing	WHMIS / SDS requirements	Review SDS before use; do not mix bleach with other cleaners
Sharps handling (lancets, syringes)	Nitrile gloves; puncture-resistant container for disposal	OHS Reg. s.6.36–6.38	Never recap by hand; place directly in sharps container
Kitchen work: hot items, cutting	Oven mitts (heat); cut-resistant gloves (cutting)	ANSI/ISEA 105	Assess task; youth involvement supervised at all times
Outdoor maintenance: mowing, gardening	Safety glasses; work gloves; hearing protection (if mowing)	CSA	Steel-toe footwear where crush risk present
Respiratory illness / droplet precautions	Procedural face mask or N95 (if airborne precautions)	ASTM F2100 / NIOSH	Per facility outbreak or public health direction
Driving / vehicle operation	Seatbelt; CSA helmet for cycling if applicable	BC Motor Vehicle Act	See Section 19 for full vehicle safety requirements

15.3 PPE Supply, Maintenance, and Replacement

- All required PPE is provided by the organization at no cost to the worker and is available in sufficient quantities at each facility at all times
- Workers inspect PPE before each use; damaged, torn, or degraded PPE is removed from service immediately and replaced
- Single-use PPE (disposable gloves, masks) is discarded after each use; it is never washed and reused

- Reusable PPE is cleaned and stored per manufacturer instructions; cleaning is the organization's responsibility, not the worker's personal responsibility
- Workers who require specialized PPE due to a medical condition or allergy (e.g., latex allergy requiring nitrile alternatives) inform the supervisor; the organization provides an appropriate substitute

15.4 PPE Training

All workers who are required to use PPE receive training covering: which PPE is required for their assigned tasks and why; how to properly don, doff, and wear each item; how to inspect PPE for damage before use; how to clean and store reusable PPE; and how to dispose of contaminated PPE safely. Training is documented and retained. PPE refresher training is provided when: a new PPE type is introduced; an incident indicates incorrect use; or a worker requests a refresher.

16. WHMIS AND HAZARDOUS PRODUCTS

Regulatory Authority: OHS Regulation Part 5; Hazardous Products Act (Canada); WHMIS 2015; WorkSafeBC WHMIS Guidelines

16.1 WHMIS 2015 Overview

The Workplace Hazardous Materials Information System (WHMIS) is Canada's national hazard communication standard for products used in workplaces. WHMIS 2015 applies to all hazardous products used at Evergreen Community Care, including cleaning products, disinfectants, medications, pest control products, and any other chemicals.

16.2 Hazardous Products Used at Evergreen Community Care

Product Category	Examples
Cleaning and disinfecting agents	Bleach (sodium hypochlorite), quaternary ammonium disinfectants, multi-surface cleaners
Medications (WHMIS-controlled)	Bulk medication storage where applicable; compounded medications; cytotoxic agents
Pest control products	Ant bait, insecticide spray
Cooking and kitchen chemicals	Oven cleaner, drain cleaner
Vehicle care	Windshield washer fluid, brake fluid
Maintenance products	WD-40, paint, caulking, solvents

16.3 Safety Data Sheet (SDS) Requirements

- A current Safety Data Sheet (SDS) must be available for every hazardous product used at each facility
- SDS binders are maintained at each facility and available to all workers at all times — including during all shifts; the Health and Safety Coordinator maintains the master SDS file
- SDSs must not be more than 3 years old; the Health and Safety Coordinator updates SDSs when products change and reviews all SDSs annually
- Workers are shown the location of the SDS binder during orientation and trained on how to read an SDS

16.4 Product Labeling

- All hazardous products must remain in their original labeled containers; transferred or decanted products are labeled with the product name, hazard pictograms, and supplier information
- Damaged, illegible, or missing labels are replaced immediately; products with unreadable labels are taken out of service until re-labeled
- WHMIS hazard pictograms posted near the chemical storage area provide a quick visual reference to the hazard categories present

16.5 Storage and Handling

- Hazardous products are stored in a locked cupboard or room, separate from food, beverages, and medications; youth do not have access to chemical storage areas
- Cleaning products are never mixed; staff are specifically trained on the danger of mixing bleach with ammonia-based products or acids

- Adequate ventilation is provided when using products with significant vapour hazard; fans or open windows are used as required by the SDS
- Minimum quantities of hazardous products are kept on-site; bulk storage is avoided

16.6 WHMIS Training

All workers receive WHMIS orientation before working with any hazardous product. Training covers: the WHMIS 2015 system; hazard classes and pictograms; how to read and use an SDS; proper labeling requirements; safe use, storage, and disposal of hazardous products in their role; and what to do in case of exposure. WHMIS training is renewed every 3 years and whenever a new hazardous product is introduced to the workplace.

17. ERGONOMICS AND MUSCULOSKELETAL INJURY PREVENTION

Regulatory Authority: OHS Regulation Part 4.46–4.53; WorkSafeBC Preventing Musculoskeletal Injury Guidelines

17.1 Risk Identification

Musculoskeletal injuries (MSIs) are among the most common workplace injuries in residential care. Key MSI risk factors in Evergreen Community Care's environment include:

MSI Risk Factor	Residential Care Examples
Forceful exertions	Manual lifting of youth; repositioning youth in bed; carrying equipment or supplies
Awkward postures	Bending to assist youth with floor-level activities; working in confined spaces; non-neutral wrist and back positions
Repetitive motion	Repetitive charting; repeated transfer assists; repeated food preparation tasks
Contact stress	Leaning or pressing body weight against hard edges during youth support activities
Whole-body vibration	Extended vehicle driving; transport over rough terrain
Prolonged static posture	Standing for extended periods during supervision; sitting without adequate breaks during documentation
Working in limited space	Small bathrooms for personal care assistance; narrow hallways

17.2 Control Measures

- Mechanical assists: mechanical lifts, slide sheets, transfer boards, and gait belts are available at facilities where youth require physical transfer assistance; their use is documented in the Individual Service Plan
- Two-person assists: any manual transfer or repositioning of a youth that requires significant force is performed by two workers using a coordinated technique
- Training: all workers receive safe lifting and ergonomics training within 60 days of hire; technique is reinforced during annual OHS training
- Workstation setup: staff work areas (desks, computers, chairs) are assessed for ergonomic fit; workers are encouraged to adjust equipment to their needs and to request assessment if discomfort persists
- Break rotation: supervisors schedule regular rest breaks and rotation between tasks to reduce repetitive strain; workers are encouraged to stretch and change posture throughout the shift
- Early reporting: workers report early signs of MSI (pain, tingling, numbness, weakness) to the supervisor before they become disabling; early intervention is more effective than waiting

17.3 MSI Incident Investigation

All reported MSI events are investigated to identify the contributing work factors. Investigations consider: task design; workplace layout; equipment adequacy; training; supervision; and worker-reported discomfort history. Findings are reported to the JHSC and corrective actions tracked to completion.

18. WORKING ALONE OR IN ISOLATION

Regulatory Authority: OHS Regulation s.4.21–4.24; WorkSafeBC Working Alone or in Isolation Guidelines

18.1 Definition and Application

A worker is working alone or in isolation when they are at a worksite by themselves and assistance is not readily available if needed. In residential care, this includes: single-staff overnight shifts; staff working in administrative areas after hours; and staff conducting outdoor activities in areas with limited sightlines or communication access.

18.2 Risk Assessment

The Program Manager conducts and documents a written risk assessment for each identified working-alone situation, considering: the nature of the tasks performed; the vulnerability of the client population present; available communication options; response time for assistance; history of incidents; and seasonal or environmental factors.

18.3 Control Measures and Check-In System

Control Measure	Detail
Scheduled check-in system	Worker contacts a designated person at defined intervals (not to exceed 2 hours); the designated contact follows the escalation protocol immediately if a check-in is missed
Communication device	A fully charged phone, two-way radio, or personal alarm is provided to the worker and confirmed functional at the start of each shift; the worker tests contact with the check-in person at shift start
Escalation protocol	If a check-in is missed: attempt phone contact x2 (2 minutes apart); contact the second designated person; if no response after 5 minutes, call 911 and provide the worker's name, last known location, and facility address
Modified scheduling	Where risk assessment indicates a specific task cannot be performed safely alone, minimum two-staff coverage is required regardless of scheduling difficulty
Environmental controls	Improved lighting; removal of blind spots; locked access to isolated areas; personal alarm systems where warranted by risk assessment

18.4 Check-In Log

Working-alone check-in contacts are logged at each facility using the designated working-alone log sheet: date, worker name, check-in time, designated contact name, and outcome. Logs are reviewed by the JHSC during monthly inspections. Systematic failures in check-in compliance are addressed as safety incidents.

18.5 Emergency Response Protocol

A written emergency response protocol is posted at each facility specifying: the working-alone contact person by shift and day; the complete escalation steps when a check-in is missed; and the chain of notification following any emergency response including police, Program Manager, and Executive Director notification. Protocols are reviewed annually and updated when staffing or communication systems change.

19. VEHICLE AND TRANSPORTATION SAFETY

Regulatory Authority: OHS Regulation Part 16; BC Motor Vehicle Act; WorkSafeBC Vehicle Safety Guidelines

19.1 Driver Authorization

Only workers who meet all of the following criteria are authorized to drive organizational vehicles or use personal vehicles for work purposes:

- Hold a valid BC driver's licence appropriate for the vehicle being driven (Class 5 minimum for passenger vehicles)
- Submit a current driver's abstract to Human Resources annually (January); abstracts showing more than two at-fault incidents or serious driving convictions in the past 3 years trigger a supervisor review before continued driving authorization
- Complete the organization's driver safety orientation before driving for work purposes
- Are not impaired by any substance, fatigue, or medical condition that affects driving ability at the time of driving

19.2 Pre-Trip Vehicle Inspection

Before each work-related trip, the driver completes a basic pre-trip inspection and documents it in the vehicle log:

Inspection Item	Standard
Fuel level	Adequate for the planned trip plus a 20% reserve
Tire condition	No visible damage, punctures, or under-inflation; tread depth adequate
Lights and signals	All operational including headlights, brake lights, turn signals, emergency flashers
Windshield and mirrors	Clean, unobstructed, and correctly adjusted
Seatbelts	Present and functional for all seating positions that will be occupied
Child restraints	Correct size for each child passenger; properly installed and adjusted
First aid kit	Present and stocked; check monthly; replenish after any use
Emergency equipment	Reflective triangles or flares present; winter kit (December–March)
Defects	Any defect that may affect safety is reported to the supervisor before the trip proceeds

19.3 Safe Driving Requirements

- Seatbelts: worn by the driver and every passenger at all times the vehicle is in motion, regardless of distance; the driver confirms all passengers are belted before moving
- Speed: observed speed limits are maximum speeds; weather, road, and traffic conditions may require driving at slower speeds
- Mobile devices: handheld mobile devices must not be used while driving; hands-free calls permitted only if essential; it is safer to pull over safely and stop
- Following distance: minimum 3-second following distance in normal conditions; 6 seconds in rain, snow, or reduced visibility
- Impairment: no alcohol within 8 hours before driving for work; no cannabis within 12 hours; prescription or over-the-counter medications that warn against driving must be disclosed to the supervisor before operating a vehicle

- Fatigue: a fatigued driver must pull over safely, notify the supervisor, and arrange alternative transportation; drowsy driving is as dangerous as impaired driving

19.4 Transporting Youth — Additional Requirements

- Age and weight-appropriate child restraints (rear-facing seat, forward-facing seat, booster seat, seat belt) are used for every child and youth passenger per BC Motor Vehicle Act requirements
- A minimum of one additional staff member, besides the driver, is present when transporting more than two youth, or when any youth has a behaviour history that creates safety concerns during vehicle travel
- A transport log is completed for every trip involving a youth passenger: date; origin; destination; purpose; driver name; youth passengers by name; departure and return times
- Youth must not be left unattended in a vehicle under any circumstances, including during brief stops
- Emergency contact information for each youth passenger is carried in the vehicle during the trip

19.5 Accident and Incident Reporting

Any accident, collision, or vehicle damage involving an organizational vehicle or a worker's personal vehicle used for work must be: reported to the supervisor immediately; documented on the Vehicle Incident Report within 24 hours; reported to ICBC as required by the Motor Vehicle Act; and reported to WorkSafeBC if the worker was injured. The vehicle is not driven again until the supervisor confirms it is safe to do so.

20. WORKPLACE INSPECTIONS

Regulatory Authority: Workers Compensation Act s.125(a); OHS Regulation s.3.5

20.1 Purpose

Regular, formal workplace inspections are one of the most effective tools for proactive hazard identification. Inspections are a statutory function of the JHSC and a core element of Evergreen Community Care's OHS Program.

20.2 Inspection Schedule

Inspection Type	Frequency	Conducted By
Formal facility inspection (full facility walkthrough using Appendix C checklist)	Monthly	JHSC member(s) + facility supervisor
Fire extinguisher and alarm system check	Monthly	Facility supervisor or designate
First aid kit inventory check	Monthly	Designated first aid attendant
Vehicle safety inspection (per vehicle)	Monthly and before each trip	Driver + fleet coordinator
Playground and outdoor equipment inspection	Monthly (weekly in high-use seasons)	Facility supervisor
Kitchen equipment and hood filter check	Monthly	Facility supervisor
Medication storage and controlled substance count verification	Each shift change	Outgoing and incoming shift lead
Working-alone check-in log review	Monthly (JHSC inspection)	JHSC inspector
Annual comprehensive facility audit	Annually	Health and Safety Coordinator + JHSC + external review

20.3 Inspection Process

- Inspectors use the Monthly Workplace Inspection Checklist (Appendix C) and supplement it with observation of any conditions not covered by the checklist
- Deficiencies are documented with: location; description of the hazard; assessed risk level; recommended corrective action; and target completion date
- Immediate hazards (Critical or High per the risk matrix) are corrected on the spot where possible; if immediate correction is not possible, the area is barricaded or the task suspended and the supervisor notified immediately
- All other deficiencies are entered into the corrective action tracking log with an assigned owner and target date
- Completed inspection reports are signed by the inspector(s) and facility supervisor and submitted to the Health and Safety Coordinator
- Inspection reports are reviewed at the next JHSC meeting; open corrective actions are tracked at every subsequent meeting until closed

20.4 Corrective Action Follow-Up

The Health and Safety Coordinator maintains a central corrective action log tracking all open items from inspections, investigations, and JHSC recommendations. Overdue corrective actions are escalated to the Program Manager and, if unresolved within 14 days of the target date, to the Executive Director. Repeat deficiencies at the same location trigger a root-cause review to identify systemic issues.

21. HEALTH AND SAFETY TRAINING MATRIX

Regulatory Authority: OHS Regulation s.3.22–3.24; WorkSafeBC Training and Certification Requirements

21.1 Mandatory OHS Training

Training	Audience	Initial Timing	Renewal
OHS Program orientation (this binder)	All workers	Before first shift	Review at each annual update; document receipt
Site-specific hazard orientation	All workers	Before first unsupervised shift at any new site	When assigned to a new facility
WHMIS 2015	All workers	Before working with any hazardous product	Every 3 years or when new products introduced
Occupational First Aid Level C (or higher)	Minimum 1 designated attendant per shift per facility	Before assuming FAA duty	Every 3 years (Level C certification cycle)
CPR / AED	All direct-care staff	Within 30 days of hire	Annual
Fire extinguisher use and fire safety	All workers	Within 30 days of hire	Annual (with evacuation drill)
Workplace Violence Prevention orientation	All workers	Before first unsupervised shift	Annual
Non-Violent Crisis Intervention (NVCI)	All direct-care staff	Within 90 days of hire; before solo shifts	Annual recertification
Trauma-informed care	All direct-care staff	Within 60 days of hire	Every 2 years or when program updates occur
Medication Administration	Direct-care staff with medication duties	Before administering any medication	Annual competency check
Standard Precautions / Infection Control	All direct-care staff	Before first shift; included in orientation	Annual refresher
Blood / bodily fluid exposure response	All direct-care staff	Before first shift	Annual
Safe lifting and ergonomics	All workers	Within 60 days of hire	Every 2 years
Driver safety / defensive driving	Workers who drive for work	Before first work-related driving duty	Every 3 years
JHSC certification (8 hours)	All JHSC members	Within 6 months of joining committee	Not required to repeat; update training encouraged
Working alone safety	Workers in identified working-alone situations	Before first solo shift	Annually; also when procedures change

21.2 Training Compliance Monitoring

- The Health and Safety Coordinator maintains a centralized OHS training compliance tracker, updated in real time as training is completed
- Supervisors receive a monthly compliance report for their team; they are responsible for scheduling overdue training promptly

- Workers with overdue safety-critical training (NVCI, First Aid, WHMIS) are restricted from duties requiring that training until compliance is restored
- Training completion is verified by: sign-in sheets; certificates of completion; and supervisor confirmation of competency demonstration where applicable
- Training records are retained for the duration of employment plus 7 years

22. RETURN-TO-WORK AND ACCOMMODATION PROGRAM

Regulatory Authority: Workers Compensation Act s.152–153; OHS Regulation s.3.8; BC Human Rights Code s.13

22.1 Purpose and Philosophy

Evergreen Community Care is committed to supporting workers who are injured or become ill — whether work-related or not — in returning to safe, productive work as early as medically appropriate. Early, supportive return to work benefits the worker's recovery, maintains workplace relationships, and supports the organization's operational continuity.

22.2 Return-to-Work Coordinator

A designated Return-to-Work Coordinator (the Health and Safety Coordinator or HR Director as assigned) manages all return-to-work planning. The Coordinator liaises with the worker, treating physician, supervisor, and WorkSafeBC for work-related claims throughout the return-to-work process.

22.3 Process Overview

Phase	Actions
Initial contact (within 24–48 hours)	Supportive contact with injured/ill worker by supervisor or Coordinator, focused on care and connection — not pressure to return; information about WorkSafeBC claims provided if work-related
Ongoing support (during recovery)	Regular supportive contact respecting medical privacy; check-in frequency agreed with the worker; WorkSafeBC reporting obligations met
Return planning	Functional abilities information requested from treating physician (functional limitations, not diagnosis); return date and capacity discussed collaboratively
Graduated return plan	Written plan developed with the worker, supervisor, and Coordinator; specifies modified duties, hours, and timeline; reviewed with updated medical information at agreed intervals
Full return	Worker transitions to full regular duties once medically cleared; formal close-out meeting with supervisor and Coordinator

22.4 Modified Duties

- Modified duties are identified in advance where possible and maintained as a standing list per facility; meaningful work is assigned, not make-work
- Modified duty assignments are temporary and not used to penalize the worker for being injured
- Modified duties respect the worker's functional limitations; they are never assigned tasks that conflict with medical restrictions
- If no suitable modified duties exist at the worker's regular facility, alternate modified work at another location is considered before unpaid leave

22.5 WorkSafeBC Claims Cooperation

- Evergreen Community Care cooperates fully and promptly with WorkSafeBC claims for work-related injuries and illnesses
- Workers are informed of their right to file a WorkSafeBC claim; the organization never discourages, delays, or obstructs legitimate claim filing

- Employer's Report of Injury or Occupational Disease (Form 7) is submitted to WorkSafeBC within 3 days of learning of a claimable injury
- Wage loss and claim documentation are submitted within required WorkSafeBC timelines

22.6 Human Rights and Duty to Accommodate

Where a worker's injury, illness, or disability triggers the duty to accommodate under the BC Human Rights Code, the return-to-work process is integrated with the Accommodation Policy (HR Policy Binder s.9D). The organization accommodates the worker to the point of undue hardship, which is determined in consultation with legal counsel and documented in writing before any accommodation refusal.

23. OHS RECORDS MANAGEMENT

Regulatory Authority: OHS Regulation s.3.16; WCA s.171; FOIPPA; PIPA

23.1 Required OHS Records

Record Type	Minimum Retention Period
Incident and near-miss investigation reports	10 years from date of report
WorkSafeBC claim files and correspondence	Per WorkSafeBC requirements; minimum 10 years
JHSC meeting minutes	2 years posted and accessible; archived permanently
Workplace inspection reports and corrective action logs	3 years; current year always accessible to all workers
Worker training records and certifications	Duration of employment + 7 years
First Aid Record book (or electronic equivalent)	3 years
Exposure records: chemical, biological, physical	30 years minimum for designated substance exposures (OHS Reg. s.5.57); general: 10 years
Hazard assessments and control plans	Current version accessible; prior versions archived 3 years after replacement
Working-alone check-in logs	3 years
PPE inspection and maintenance records	3 years
Safe work procedures (all versions)	Current version + all prior versions: 3 years after replacement
WHMIS training records and SDS binder update log	Duration of employment + 7 years
Emergency drill records	3 years
Vehicle inspection logs and incident reports	7 years
Return-to-work plans and modified duty records	10 years

23.2 Record Access

- Workers may access their own training, exposure, and incident records upon written request to the Health and Safety Coordinator; records are provided within 10 business days
- JHSC meeting minutes are posted at each facility within 7 days of each meeting and remain posted until superseded by the following month's minutes
- WorkSafeBC and other authorized regulators are provided access to records as required by law and without delay

23.3 Confidentiality

OHS records containing personal medical information are stored separately from general personnel files with access restricted to the Health and Safety Coordinator, HR Director, and Executive Director.

Medical information is shared with supervisors only on a need-to-know basis for the purposes of modified duty planning, and limited to functional limitations, not diagnosis. PIPA and FOIPPA obligations apply to all OHS records.

24. PROGRAM EVALUATION AND CONTINUOUS IMPROVEMENT

Regulatory Authority: OHS Regulation s.3.1; WorkSafeBC Program Evaluation Guidelines

24.1 Annual OHS Program Review

The OHS Program is formally reviewed annually by the Health and Safety Coordinator, the JHSC, and the Executive Director each June, in advance of the July 1 policy update cycle. The review assesses effectiveness against the year's KPIs, identifies gaps or emerging hazards, and sets priorities and actions for the coming year.

24.2 Key Performance Indicators

Key Performance Indicator	Annual Target
Lost-time injury frequency rate (LTIFR)	Year-over-year reduction; organizational benchmark established in Year 1
Medical aid frequency rate	Year-over-year reduction
Near-miss reports submitted	Increase year-over-year (higher reporting reflects stronger safety culture)
Incident investigation completion within required timeline	100%
WorkSafeBC reportable incidents notified within required timeframe	100%
Mandatory OHS training compliance — all workers	100%
Monthly workplace inspections completed	100% (12 of 12 per facility)
Corrective actions closed within target date	90% or greater
JHSC meetings held as scheduled	100%
Workplace violence incidents resulting in lost time	Year-over-year reduction
Worker satisfaction with OHS program (annual survey)	70% or greater rating of Satisfied or Very Satisfied

24.3 Annual OHS Performance Report

The Health and Safety Coordinator prepares an Annual OHS Performance Report each August summarizing: incident statistics; trend analysis; inspection results; training compliance; JHSC activity; progress on prior-year corrective actions; and priorities for the coming year. The report is presented to the Board of Directors and shared with all staff through team meetings and the OHS bulletin board.

24.4 External Review

- Evergreen Community Care welcomes and fully cooperates with any WorkSafeBC inspection, consultation, or audit
- A WorkSafeBC order or inspection finding triggers an immediate written corrective action plan, with target completion dates for each order item

- An independent external OHS audit is conducted at minimum every 3 years to provide objective assessment of program effectiveness and legislative compliance

25. POLICY REVIEW AND APPROVAL

25.1 Review Triggers

This OHS Program is reviewed and updated in the following circumstances:

- Annual review: July 1 each year, aligned with the HR Policy Binder review cycle
- Legislative amendment: whenever the Workers Compensation Act, OHS Regulation, or WorkSafeBC policies are significantly amended in ways affecting residential care operations
- Serious incident: following any workplace fatality, critical injury, or serious workplace violence incident
- Significant operational change: new facility, new client population, new program type, or major staffing model change
- JHSC or WorkSafeBC recommendation: at any time the JHSC or WorkSafeBC identifies a gap or required update

25.2 Approval Authority and Version Control

Role	Responsibility
Policy Owner	Health and Safety Coordinator — drafts updates; coordinates JHSC review
JHSC Review	Both worker and management co-chairs review and sign each updated version before it goes to Executive Director
Approval Authority	Executive Director
Board Notification	Board of Directors is informed of significant changes at the next board meeting following approval
Distribution	Posted on OHS bulletin boards at all facilities; all workers notified through team meetings; superseded versions archived with the date of replacement noted

25.3 Version History

Version	Date	Summary of Changes
1.0	July 1, 2026	Initial OHS Program document — 20 sections
2.0	July 1, 2026	S1. OCCUPATIONAL DISEASE PREVENTION PROGRAM <i>Regulatory Authority: OHS Regulation Part 5; Workers Compensation Act s.5; WorkSafeBC Occupational Disease Guidelines</i> S1.1 Definition and Scope An occupational disease is a disease, illness, or condition caused by or significantly contributed to by conditions of the worker's employment. In residential care settings, the primary occupational disease risks include: infectious disease; hearing loss from prolonged noise exposure; skin disorders from frequent handwashing and chemical exposure; respiratory conditions from

chemical inhalation; and musculoskeletal disorders.

S1.2 Infectious Disease Prevention

Residential care environments carry elevated risk of infectious disease transmission between youth, and from youth to staff. Evergreen Community Care implements a comprehensive infection prevention and control program as follows:

- Immunization: staff are encouraged to be current on all recommended immunizations including annual influenza, Hepatitis B series, Tdap, and COVID-19 as per BC Centre for Disease Control guidance; the organization facilitates access to publicly funded immunization programs
- Respiratory illness management: staff with active respiratory infections (fever, productive cough) are excluded from work until symptom resolution per public health guidance; surgical masks are available for staff with mild respiratory symptoms who must attend work
- Gastrointestinal illness management: staff with vomiting or diarrhea are excluded from work for a minimum of 48 hours after symptoms resolve; notification to the Program Manager is required so enhanced cleaning can be initiated
- Outbreak management: when two or more cases of similar illness occur in the facility within 7 days, the Program Manager notifies the local public health authority (Fraser Health or applicable regional authority) and follows their direction; MCFD is also notified per the Residential Care Critical Incident Reporting requirements
- Bloodborne pathogen exposure: the Blood and Bodily Fluid Exposure Response procedure (Section 6.4) is followed immediately; WorkSafeBC Form 7 is filed within 3 days if medical treatment beyond first aid is required

S1.3 Hearing Conservation

Workers in residential care environments may be exposed to elevated noise levels from youth distress behaviours, alarm systems, and general residential activity. Where noise levels are identified as a concern:

- The Program Manager assesses facility noise levels and documents the assessment
- Where sustained noise levels approach or exceed 85 dB(A), a formal noise assessment by a qualified professional is conducted per OHS Regulation s.7.1
- Hearing protection (foam earplugs, earmuffs) is made available where elevated noise exposure is identified
- Workers who regularly work in elevated-noise environments are offered audiometric testing as a baseline and at defined intervals per OHS Regulation requirements

S1.4 Skin Disorder Prevention

Frequent handwashing, glove use, and exposure to cleaning chemicals create significant risk of occupational dermatitis. Prevention measures include:

- Alcohol-based hand sanitizer is available as an alternative to soap and

water where hands are not visibly soiled; repeated handwashing with harsh soap is a significant dermatitis risk

- Moisturizing skin cream is available at all handwashing stations; application after each handwashing is encouraged
- Gloves are the correct fit for each worker to prevent friction and sweat-related skin breakdown
- Workers who develop persistent skin irritation, rash, or dryness are referred to occupational health or their physician; early reporting prevents chronic skin conditions

S2. SAFE WORK PROCEDURE — PHYSICAL INTERVENTION AND RESTRICTIVE PRACTICES

Regulatory Authority: OHS Regulation s.4.27–4.31; WCA s.115; Residential Care Regulation s.74–78; MCFD Standards

S2.1 Governing Principles

Physical intervention in residential care is a last resort employed only when a youth poses an imminent risk of serious harm to themselves or others, and when de-escalation and less restrictive approaches have been exhausted or are not feasible in the time available. Every physical intervention carries risk of physical injury to both the youth and the staff involved. Evergreen Community Care adopts a philosophy of minimal force, minimal duration, and maximum dignity.

S2.2 Authorization and Training

- Only staff who hold a current, valid Non-Violent Crisis Intervention (NVCi) or equivalent certification may use approved physical intervention techniques
- Certification is valid for 12 months; expired certification removes authorization to use physical intervention until recertified
- Approved techniques are limited to those taught in the current NVCi curriculum; improvised or untrained techniques are strictly prohibited
- The use of prone restraint (face-down restraint) is prohibited in all circumstances
- The use of any technique that restricts breathing, applies pressure to the neck or chest, or causes pain compliance is prohibited

S2.3 Before Physical Intervention — Decision Framework

Before using physical intervention, the staff member confirms:

- Is there an imminent risk of serious harm? (Not just challenging behaviour — serious harm)
- Has de-escalation been attempted? (Space, voice, distraction, redirection, environmental modification)
- Is there a safer alternative? (Removal of others from the area; wait for backup; environmental barrier)
- Is backup available? (A solo physical intervention without backup is higher risk and should be avoided where at all possible)

S2.4 During Physical Intervention

- Use only the minimum level of intervention required to prevent the harm
- Verbally communicate with the youth throughout: identify yourself; explain what is happening; reassure the youth that they are safe
- Monitor the youth continuously for signs of distress: skin colour changes; laboured breathing; loss of consciousness; verbal indicators of pain or medical concern
- Release the intervention as soon as the immediate risk of serious harm has passed; the intervention does not continue as a consequence or punishment
- Minimum two staff for any physical intervention; the non-restraining staff member monitors the youth and is ready to call 911 if any medical concern arises

S2.5 Post-Intervention Requirements

- Conduct a medical wellness check of the youth immediately following any physical intervention; call 911 if there is any injury, complaint of pain, or indication of respiratory distress
- Allow the youth time to regulate before attempting a verbal debrief; a calm reintegration is more therapeutic than an immediate discussion of the behaviour
- Document the physical intervention on the Incident Report form within 2 hours: specific techniques used; duration; position of youth; staff involved; youth's response; any injuries or medical concerns; MCFD notification status
- Notify the Program Manager immediately; the Program Manager notifies the MCFD social worker within the required Critical Incident timeline
- Offer all involved staff a debrief with the supervisor before the end of the shift; EFAP referral for any staff member who appears significantly distressed by the event

S3. OHS COMMUNICATION AND WORKER CONSULTATION PROGRAM

Regulatory Authority: Workers Compensation Act s.119–122; OHS Regulation s.3.26–3.28

S3.1 OHS Bulletin Board Requirements

Every Evergreen Community Care facility maintains a dedicated OHS Bulletin Board in a prominent location accessible to all workers (commonly the staff room or main hallway). The bulletin board must contain the following items at all times, kept current:

- WorkSafeBC “In Case of Injury” poster (current BC version) — legally required posting
- Names and contact information of JHSC members (worker and management co-chairs and all members)
- JHSC meeting minutes from the most recent meeting
- Summary of open JHSC corrective actions and their status

- Emergency contacts: 911; Program Manager on-call; MCFD after-hours; Poison Control (1-800-567-8911); WorkSafeBC (1-888-967-5377)
- Facility emergency evacuation map showing exits, assembly point, fire extinguisher locations, and first aid kit location
- Current copy of Evergreen Community Care's Health and Safety Policy Statement (Section 1.1)
- WorkSafeBC order or direction (if any currently in effect)

S3.2 Communicating OHS Information to Workers

- New OHS Program changes or updates are communicated to all affected workers within 14 days through: team meetings; posted notice on the OHS bulletin board; and direct supervisor communication for high-priority changes
- Critical incident learnings (anonymized where appropriate) are shared with all staff at the next team meeting following the incident investigation, as part of a continuous safety learning culture
- WorkSafeBC orders, notices, and inspection reports affecting any facility are posted on the OHS bulletin board within 24 hours of receipt and remain posted until the order is resolved
- Annual OHS performance data (see Appendix J) is shared with all workers each fall; the sharing process includes a brief team discussion of priorities for the coming year

S3.3 Worker Hazard Reports

Any worker may submit a hazard report at any time — in writing, verbally to their supervisor, or through the JHSC worker representative. Hazard reports are responded to as follows:

- The supervisor acknowledges receipt of every hazard report in writing (including by email or text) within 24 hours
- The supervisor or Program Manager investigates the reported hazard within 7 days and documents findings
- The worker is informed of the investigation outcome and any corrective action in writing within 14 days
- Hazard reports and their outcomes are reported to the JHSC at the next monthly meeting
- A worker who submits a hazard report receives no negative consequence for doing so; anonymous hazard reports are accepted where a worker wishes to remain unidentified

S4. EXTREME TEMPERATURE WORK PROCEDURES

Regulatory Authority: OHS Regulation s.4.80–4.86 (heat stress); WorkSafeBC Working in Extreme Heat Guidelines

S4.1 Heat Stress Prevention

Heat illness (heat exhaustion and heat stroke) is a serious occupational health hazard that can be fatal. In residential care, staff may face heat exposure

during: outdoor activities in summer; indoor environments without adequate cooling; and physically demanding tasks (personal care, physical intervention) in warm conditions. When the humidex exceeds 35 degrees Celsius:

- Mandatory cool-down breaks of 10 minutes per hour are provided to all workers performing moderate to high physical activity outdoors or in hot indoor environments
- Cool water is available and easily accessible to all workers and youth throughout the shift
- Outdoor activities are rescheduled to early morning or late afternoon when temperatures are lower, or cancelled entirely when humidex exceeds 40 degrees
- Windows, fans, and air conditioning (where available) are maximized; cross-ventilation is optimized
- Staff monitor themselves and each other for signs of heat illness: excessive sweating; weakness; dizziness; nausea; confusion; hot dry skin (heat stroke emergency)

S4.2 Heat Stroke Emergency Response

Heat stroke (body temperature above 40 degrees Celsius; confusion; hot dry skin; no sweating) is a medical emergency:

- Call 911 immediately
- Move the person to a cool location immediately
- Apply cool (not ice cold) water or wet cloths to the skin; fan the person
- Do not give fluids to an unconscious person
- Monitor breathing and be prepared to perform CPR if breathing stops

S4.3 Cold Stress Prevention

Workers performing outdoor tasks in cold weather (temperatures below 5 degrees Celsius or with significant wind chill) are protected as follows:

- Adequate layered clothing including windproof outer layer, insulating mid-layer, and moisture-wicking base layer is available to workers who must perform outdoor tasks in cold weather
- Outdoor tasks in extreme cold (wind chill below -20 degrees Celsius) are limited in duration with mandatory warm-up breaks in a heated area
- Workers and youth are monitored for signs of hypothermia (shivering, confusion, slow speech, drowsiness) and frostbite (numbness, white or grayish skin)
- Emergency warm-up protocol: any person showing signs of hypothermia is moved to a warm environment immediately; 911 is called for moderate or severe hypothermia

S5. SHIFT WORK AND FATIGUE RISK MANAGEMENT

Regulatory Authority: OHS Regulation s.4.49; WorkSafeBC Fatigue Guidelines; WCA s.115(2)
(c)

S5.1 Fatigue as an OHS Hazard

Fatigue is a recognized occupational hazard in residential care, where 24-hour operations require shift work including evenings, nights, weekends, and statutory holidays. Fatigued workers demonstrate impaired judgment, slowed reaction time, reduced situational awareness, and increased likelihood of both workplace injuries and errors in care. Evergreen Community Care treats fatigue risk management as a formal component of its OHS Program.

S5.2 Scheduling Protections

- Minimum 8 consecutive hours free from work between scheduled shifts (11 hours is the preferred standard to allow for commute and sleep preparation time)
- No worker is scheduled for more than 5 consecutive shifts without a minimum 24-hour break, except by mutual agreement and with supervisor monitoring of fatigue indicators
- Double shifts (16 consecutive hours) are limited to genuine emergency coverage only; a worker who has completed a double shift is not scheduled for the following shift without minimum 12 hours rest
- Night shift workers are not scheduled for daytime commitments (training, meetings) that conflict with their rest window without the worker's advance written agreement
- Schedules are published a minimum of 2 weeks in advance to allow workers to plan adequate sleep and rest

S5.3 Worker Responsibilities

- Workers are responsible for arriving at work adequately rested and fit for duty; arriving excessively fatigued is a fitness-for-duty issue governed by Section 9 of this Program
- Workers who are aware they will arrive at work significantly fatigued (due to a family emergency, illness, or unavoidable circumstance) should notify their supervisor as early as possible so alternate coverage can be considered
- Workers may decline a voluntary overtime shift without negative consequence where acceptance would compromise their ability to arrive at their next scheduled shift adequately rested

S5.4 Supervisor Monitoring

- Supervisors actively monitor for fatigue indicators in their team: declining quality of documentation; repeated small errors; irritability or withdrawal; reports of difficulty staying awake
- A supervisor who observes significant fatigue indicators in a worker addresses them in a private, supportive conversation and modifies the worker's duties or sends them home if safety requires it
- Overtime approval by supervisors considers the cumulative shift load of the worker being asked to stay, not just the immediate staffing need

S6. PREGNANCY AND REPRODUCTIVE HEALTH IN THE WORKPLACE

Regulatory Authority: OHS Regulation s.4.09; BC Human Rights Code s.13; Employment Standards Act Part 6

S6.1 Hazard Assessment for Pregnant and Breastfeeding Workers

When a worker notifies Evergreen Community Care that they are pregnant, may become pregnant, or are breastfeeding, the organization conducts a workplace hazard assessment specific to that worker's role and condition. This is a proactive health and safety obligation, not dependent on the worker requesting it. The assessment considers:

- Biological hazards: exposure to communicable diseases, bloodborne pathogens, or cytomegalovirus (CMV) — all of which carry specific risks during pregnancy
- Physical demands: manual lifting, physical intervention, prolonged standing, heavy physical exertion
- Chemical exposures: specific cleaning products or disinfectants that may be contraindicated during pregnancy
- Psychological hazards: exposure to traumatic events and high-stress crisis situations that carry elevated risk during pregnancy
- Shift work and fatigue: implications of overnight shifts and extended hours during pregnancy

S6.2 Accommodation Obligations

Where the hazard assessment identifies risks that cannot be eliminated or controlled to an acceptable level for a pregnant or breastfeeding worker, Evergreen Community Care accommodates the worker to the point of undue hardship per the BC Human Rights Code. Accommodation may include: modified duties avoiding identified hazards; schedule adjustments; temporary redeployment; or, where no suitable modified duties exist, unpaid leave (with the option to access EI maternity benefits early per federal legislation). Accommodation decisions are made by HR in consultation with the worker and their healthcare provider.

S7. OFF-SITE, COMMUNITY, AND REMOTE WORK SAFETY

Regulatory Authority: OHS Regulation s.3.1; WCA s.115; WorkSafeBC Guidance on Remote Work

S7.1 Community and Off-Site Activities

When staff and youth leave the facility for community outings, medical appointments, recreational activities, or any other off-site purpose, the OHS Program continues to apply. The on-duty staff member is responsible for maintaining a safe environment for both themselves and the youth in their care throughout the activity.

- Pre-outing hazard assessment: the lead staff member assesses the outing destination for known hazards (traffic, water, crowd density, equipment safety) before departure and addresses any concerns with the supervisor

- Communication: the lead staff member ensures they have a functioning phone, the facility's main number, the supervisor's direct number, and emergency contact information for each youth participant throughout the outing
- Medical emergency: 911 is called immediately for any medical emergency during an off-site activity; the supervisor is notified simultaneously or immediately after; the outing is ended and the group returns to the facility when it is safe to do so
- Violent incident during a community outing: staff withdraw themselves and youth from the immediate vicinity; call 911 if there is an ongoing threat; notify the supervisor; complete an Incident Report on return

S7.2 Administrative and Remote Work

Where administrative staff or managers perform work from a home office or remote location, Evergreen Community Care extends the following OHS obligations:

- Workstation ergonomics: remote workers are provided guidance on setting up an ergonomically appropriate home workstation; significant ergonomic concerns are addressed through the Accommodation Policy
- Work-related injury: injuries that occur during work activities in a home office remain eligible for WorkSafeBC coverage if they arise out of and in the course of employment
- Mental health: the isolation and boundary challenges of remote work are recognized as psychosocial hazards; EFAP access and regular supervisor check-ins apply equally to remote workers

S8. WORKSAFEBC REGULATORY REFERENCE SUMMARY FOR RESIDENTIAL CARE

The following table provides a quick reference to the key sections of the Workers Compensation Act (WCA) and OHS Regulation most directly applicable to Evergreen Community Care's residential care operations. This is a reference summary only — the full text of all regulations is available at [worksafebc.com](https://www.worksafebc.com).

Regulation Reference	Subject	Key Requirements Care
WCA s.115	Employer duties	Provide safe system of work; inform workers of risks; ensure compliance with OHS Regulation
WCA s.116	Worker duties	Follow safe work procedures; report hazards; use PPE; protect others
WCA s.119	Prime contractor	Where two or more contractors on one worksite; must establish OHS coordination
WCA s.125–135	JHSC requirements	Mandatory for 20+ workers; monthly meetings; member rights and functions

WCA s.3.12 (OHS Reg)	Right to refuse	Refuse undue hazard; investigation steps; no reprisal
WCA s.172–176	Incident reporting	Employer reporting obligations; investigation requirements; record-keeping
OHS Reg. s.3.1	OHS Program	Written program required for employers with 20+ workers; annual review
OHS Reg. s.3.22	New worker orientation	Required before unsupervised work; specific content requirements
OHS Reg. s.4.1–4.3	Hazard identification	Proactive hazard ID and risk assessment; document and control
OHS Reg. s.4.20	Impairment	Workers must not be impaired; employer must address impairment
OHS Reg. s.4.21–4.24	Working alone	Written procedures for check-in and emergency response
OHS Reg. s.4.24	Psychological safety	Employer must address risk of work-related psychological harm
OHS Reg. s.4.27–4.31	Violence prevention	Written program; risk assessment; post-incident support
OHS Reg. s.4.46–4.53	Ergonomics / MSI	Identify and control MSI risk factors
OHS Reg. Part 5	WHMIS	SDS requirements; labeling; training; annual update
OHS Reg. Part 6	Biological agents	Standard precautions; sharps handling; exposure response
OHS Reg. Part 7	Noise	Assess and control noise above action level (85 dB); hearing protection
OHS Reg. Part 8	PPE	Employer provides; worker uses; inspect and maintain
OHS Reg. Part 16	Mobile equipment	Vehicle inspection; operator competency; passenger safety
OHS Reg. s.7.1–7.19	First aid	Supplies required by workforce size; certified attendant on each shift; records
WorkSafeBC Policy D3-115-2	Workplace bullying	Written policy; annual training; investigation process; no reprisal
WCA s.5.1	Mental disorder claims	Compensable if caused by work; employer cooperation required
WCA s.152–153	RTW obligations	Employer must offer suitable work; cooperate with WorkSafeBC RTW process

S9. FIRST AID PROGRAM REQUIREMENTS

Regulatory Authority: OHS Regulation Part 7 (s.7.1–7.42); WorkSafeBC First Aid Regulation

S9.1 First Aid Attendant Requirements

Evergreen Community Care is required to provide first aid coverage appropriate to the number of workers on each shift and the hazard level of the work. For residential care facilities:

Workers on Shift	Required First Aid Level	Additional Requirements
1 to 9 workers, low hazard	Occupational First Aid Level 1 (OFA 1) or Basic First Aid Certificate	First Aid kit Type 1 on site
1 to 9 workers, moderate-high hazard (residential care classification)	Occupational First Aid Level 2 (OFA 2)	First Aid kit Type 2 on site
10 to 49 workers	OFA Level 2 minimum	First Aid room or designated first aid area
Any shift where a worker drives as part of their duties	At minimum a valid Emergency First Aid certificate (Level C CPR)	Vehicle first aid kit

S9.2 First Aid Supplies

- First aid kits are stocked to Type 1 or Type 2 specifications per OHS Regulation s.7.15 based on workforce size and hazard level
- First aid kits are inspected and inventoried monthly by the designated first aid attendant; inspection is documented and deficiencies corrected before the end of the same shift
- Supplies used from the first aid kit are restocked within 24 hours of use
- The location of the first aid kit at each facility is posted on the OHS bulletin board, the emergency evacuation map, and near the kit itself
- An AED (Automated External Defibrillator) is recommended for facilities where cardiac emergency response time from EMS may exceed 4 minutes; where an AED is present, designated staff receive AED training

S9.3 First Aid Records

A First Aid Record book (or approved electronic equivalent) is maintained at each facility. An entry must be made for every instance of first aid rendered, including: name and contact of the injured person; date and time; nature and location of injury or illness; first aid treatment provided; name of the first aid attendant who provided care; and whether the person was referred for medical treatment. First Aid Records are confidential health records and are retained for a minimum of 3 years per OHS Regulation s.7.26.

Comprehensive expansion to 26 sections + 10 appendices; new sections on psychological safety (s.8), ergonomics (s.17), vehicle safety (s.19), WHMIS (s.16), working alone (s.18), contractor safety (s.14); full WorkSafeBC regulation reference throughout

26. SIGNATURES AND APPROVAL

This Occupational Health and Safety Program has been developed in good faith to meet or exceed the requirements of the Workers Compensation Act, the Occupational Health and Safety Regulation, and all applicable WorkSafeBC requirements for Evergreen Community Care's residential care operations.

By signing below, each party confirms their commitment to implementing and supporting this Program.

Role	Signature / Date
Executive Director	Signature: _____ Date: _____
Health and Safety Coordinator	Signature: _____ Date: _____
JHSC Worker Co-Chair	Signature: _____ Date: _____
JHSC Management Co-Chair	Signature: _____ Date: _____
Program Manager (Facility 1)	Signature: _____ Date: _____
Program Manager (Facility 2)	Signature: _____ Date: _____

APPENDIX A — NEW WORKER SAFETY ORIENTATION CHECKLIST

To be completed before unsupervised work begins. Both worker and supervisor sign and retain a copy. File original in personnel file.

Field	
Worker Name	
Position / Role	
Facility	
Date of Orientation	
Orienting Supervisor	

Check each item as completed:

#	Orientation Topic	✓ / N/A
1	Worker rights and responsibilities under the Workers Compensation Act (know, participate, refuse unsafe work, no reprisal)	
2	Location of this OHS Program Binder; JHSC member contacts; how to report a hazard or incident	
3	Site-specific hazard walkthrough: violence risks, chemical storage, slips/trips, evacuation routes, assembly point	
4	Workplace violence prevention: facility-specific risks; de-escalation approach; backup communication	
5	Emergency procedures: fire evacuation route and assembly point; medical emergency steps; lockdown procedure	
6	Location and use of first aid kit; name of designated first aid attendant on shift	
7	Location and use of fire extinguishers; how to call 911; fire alarm location and test procedure	
8	Safe work procedures reviewed for all tasks assigned to this worker	
9	WHMIS 2015: hazardous products used at this facility; SDS binder location; how to read an SDS	
10	PPE required for this role; location; how to don, doff, inspect, and store	
11	How to complete an Incident Report (Appendix B); importance of near-miss reporting	
12	Right to refuse unsafe work — step-by-step process (Section 10); assurance of no reprisal	
13	Infection prevention and control: standard precautions; handwashing; PPE for bodily fluid contact	
14	Safe lifting and ergonomics: correct technique; available mechanical assists; two-person lift protocol	

15	Working alone procedures for this site (if applicable): check-in system, emergency contact	
16	Medication storage location and lock; worker's role regarding medication (if applicable)	
17	Youth rights and dignity: professional boundaries; mandatory reporting obligation (CFCSA)	
18	Shift handover requirements and shift log completion	
19	OHS bulletin board location: JHSC minutes, WorkSafeBC poster, emergency contacts	
20	Worker has received copy of this OHS Program binder and been encouraged to read it	

Worker Signature / Date	Supervisor Signature / Date

APPENDIX B — INCIDENT / NEAR-MISS REPORT FORM

Complete within 24 hours of any incident. For violent incidents: within 2 hours. For critical injuries or fatalities: immediately.

Field	
Worker(s) Involved	
Date / Time of Incident	
Location (facility and specific area)	
Report Completed By	
Date Report Completed	
Supervisor Notified — Name and Time	
Witnesses	

Incident Type (check all that apply)	☐ Injury ☐ Near Miss ☐ Workplace Violence ☐ Threat ☐ Property Damage ☐ Bodily Fluid Exposure ☐ Medication Error ☐ Other: _____
--------------------------------------	--

Section	Detail
Description of incident (what happened, in sequence; use objective, factual language)	
Location and environmental conditions at time of incident	
Immediate actions taken / first aid provided	
Was the worker taken off duty? ☐ Yes ☐ No Did the worker require medical attention beyond first aid? ☐ Yes ☐ No Was the worker hospitalized? ☐ Yes ☐ No	
WorkSafeBC Notified? ☐ Yes — Date/Method: _____ ☐ No — Reason: _____	
Direct / immediate cause (what immediately caused the event)	
Root / underlying cause(s) (what system, process, or organizational factor allowed	

the immediate cause to occur)	
Contributing factors	
Corrective action(s) to prevent recurrence — Action, Owner, and Target Date	
JHSC Reviewed? ☐ Yes Date: _____ ☐ Pending — next JHSC meeting date: _____	

Investigator Signature / Date	Supervisor / Program Manager Signature / Date

APPENDIX C — MONTHLY WORKPLACE INSPECTION CHECKLIST

Field	
Facility / Location	
Inspector(s)	
Date of Inspection	
JHSC Member Present?	☐ Yes ☐ No

Inspection Item	OK	Issue	Notes / Action Required
Fire extinguishers: present, charged, pin intact, accessible, last service within 1 year	☐	☐	
Fire alarm pull stations: unobstructed and accessible	☐	☐	
Smoke detectors: test light green; test button functional	☐	☐	
Sprinkler heads (if present): unobstructed; no paint or damage	☐	☐	
Emergency exits: all exit signs illuminated; doors open freely; routes clear	☐	☐	
Evacuation route diagrams posted and current	☐	☐	
Assembly point posted and accessible	☐	☐	
First aid kit: stocked, accessible, and not expired	☐	☐	
First aid attendant name posted near kit	☐	☐	
AED (if present): indicator light green; pads not expired	☐	☐	
OHS bulletin board: WorkSafeBC poster current, JHSC minutes current, emergency contacts posted	☐	☐	
Floors: clean, dry, non-slip; no trip hazards	☐	☐	
Stairs and handrails: in good condition; handrails secure	☐	☐	
Walkways and corridors: clear of stored items and obstructions	☐	☐	
Lighting: adequate in all areas including stairwells, storage, exterior	☐	☐	
Extension cords: not used as permanent wiring; not under carpets or through doorways	☐	☐	
Electrical outlets and switches: in good condition; covers in place	☐	☐	
Electrical panel: clearly labeled; 1-metre clearance maintained	☐	☐	
Hazardous materials cabinet: locked; products labeled; SDS current and accessible	☐	☐	
Cleaning products: stored away from food and medications; labeled	☐	☐	

Medication storage: locked; controlled substances double-locked; count log current	☐	☐	
Sharps container: properly mounted; not more than 3/4 full	☐	☐	
Kitchen: stove and hood clean; no grease buildup; extinguisher accessible	☐	☐	
Refrigerator temperature: at or below 4°C (check log)	☐	☐	
Freezer temperature: at or below -18°C (check log)	☐	☐	
PPE supplies: gloves, masks, eye protection — adequate supply and accessible	☐	☐	
Youth personal areas: no fire hazards; no sharp objects accessible; reasonable cleanliness	☐	☐	
Outdoor areas: no broken equipment, glass, needles, or debris	☐	☐	
Vehicles (if applicable): logs current; first aid kit present; no visible damage	☐	☐	
Working-alone check-in log: entries complete and current	☐	☐	
JHSC recommendation from last meeting: corrective action status	☐	☐	

Inspector(s) Signature / Date	Supervisor / Program Manager Signature / Date

Deficiencies requiring corrective action are entered in the JHSC corrective action tracking log. This report is submitted to the Health and Safety Coordinator within 3 business days of completion.

APPENDIX D — WORK REFUSAL DOCUMENTATION FORM

Complete when a worker exercises the right to refuse unsafe work (WCA s.3.12). Retain for minimum 10 years.

Field	
Worker Name / Position	
Date / Time of Refusal	
Location / Task Refused	
Supervisor Notified — Name and Time	
JHSC Member Notified — Name and Time	

Section	Detail
Description of the unsafe condition or hazard as described by the worker	
Initial supervisor investigation findings	
Worker's response to supervisor's findings — satisfied ☐ / continued refusal ☐	
JHSC member / worker representative involvement — name and findings	
Interim control measures implemented (if any)	
Outcome of investigation	☐ Hazard confirmed and corrected — describe corrective action: ☐ Hazard not confirmed by supervisor and JHSC ☐ Referred to WorkSafeBC — date referred: _____
WorkSafeBC Prevention Officer findings (if involved)	
Worker reassignment during investigation (if applicable)	
Date matter resolved	

Worker Signature / Date	Supervisor Signature / Date	JHSC Representative Signature / Date

APPENDIX E — HAZARD ASSESSMENT AND CONTROL FORM

Field	
Facility / Location	
Task or Work Area Assessed	
Assessment Conducted By	
Date of Assessment	
JHSC Review Date	
Next Scheduled Review	

Hazard Description	Likelihood (1–5)	Severity (1–5)	Risk Level	Control Measures	Owner / Date

Risk Level Guide: Low = routine monitoring | Medium = corrective action within 30 days | High = corrective action within 7 days | Critical = stop work immediately

Assessor Signature / Date	Supervisor / JHSC Co-Chair Signature / Date

APPENDIX F — WORKPLACE VIOLENCE RISK ASSESSMENT TEMPLATE

Complete annually per facility, or following any serious violent incident. File with JHSC. Retain for minimum 10 years.

Facility	
Date Assessed	
Assessors	
Review Date	

Risk Factor	Risk Level (Low / Med / High)	Controls in Place / Recommended
Nature of client population: trauma history, aggression history, known triggers		
Physical layout: isolated areas, blind spots, escape route adequacy		
Access control: door locking, visitor screening, key management		
Staffing levels: single-staff coverage periods, supervisor availability		
High-risk times of day: bedtime, medication time, shift change		
History of violence incidents in past 12 months		
Communication systems: phones, alarms, personal devices		
Post-incident support: CISD access, EFAP, modified duties		
NVCI training compliance: percent of direct-care staff current		
BSP coverage: percent of youth with aggression history who have current BSP		
External visitors: family conflict history, trespass risks, unauthorized callers		

Overall Violence Risk Level	☐ Low ☐ Medium ☐ High ☐ Critical — immediate action required
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Assessor Signature / Date	Program Manager Signature / Date

APPENDIX G — WORKSAFEBC REPORTING QUICK REFERENCE

WorkSafeBC Employer Contact: 1-888-967-5377 (24 hours). Online reporting: worksafebc.com. Form 7 (Employer's Report): worksafebc.com/forms.

Event	Required Action	Timeline
Worker fatality at workplace	Phone WorkSafeBC immediately; do not disturb scene without authorization	Immediately — phone
Critical injury: unconscious, fracture of major bone, amputation, severe burn, loss of sight, internal organ damage	Phone WorkSafeBC immediately; written notice within 48 hours	Immediately by phone; 48 hrs written
Major structural failure or collapse	Phone WorkSafeBC immediately	Immediately
Serious violent incident resulting in injury requiring medical treatment beyond first aid	Form 7 within 48 hours; immediate call if critical injury resulted	48 hours (Form 7); immediately if critical
Work injury requiring medical attention beyond first aid (Employer's Report Form 7)	File Form 7 online or by mail	Within 3 days of learning of injury
WorkSafeBC mental disorder claim (worker files Form 6A)	Cooperate with claim; provide workplace information as requested	Within timelines specified by WorkSafeBC
WorkSafeBC Stop-Work Order issued	Stop work in affected area immediately; inform all workers; do not resume until order is removed in writing	Immediately on receipt
WorkSafeBC Prevention Officer requests records	Provide all requested records without delay	Immediately on request
First aid only — no lost time, no medical treatment beyond first aid	Record in First Aid Record book only; no WorkSafeBC report required	Same day; entry in FA Record book
Near-miss — no injury	Internal Incident Report only; report to JHSC	Within 24 hours (internal)

APPENDIX H — PPE SELECTION AND INSPECTION GUIDE

PPE Item	When Required	Inspect For	Replace When
Nitrile examination gloves	All contact with blood, bodily fluids, non-intact skin	Tears, holes, discoloration before putting on	Single-use: after every use; do not reuse
Nitrile utility gloves (cleaning)	Use of bleach or corrosive cleaning products	Cracks, holes, thinning at fingertips	When damaged or after 30 days of regular use
Disposable gown/apron	Risk of splashing blood or bodily fluids	Intact before use	Single-use: after every use
Procedural face mask	Droplet precautions; aerosol-generating procedures	No visible soiling or damage before use	Single-use: after every use or when moist
N95 respirator (if applicable)	Airborne precautions (e.g., suspected TB, per public health direction)	Proper seal test each time; no cracks or deformation	Per manufacturer; approximately 8 hours or sooner if damaged
Safety glasses / face shield	Risk of splashing liquids; kitchen tasks involving hot liquid	Scratches impairing vision; frame damage	When vision is impaired; after any impact
Cut-resistant gloves	High-risk cutting tasks; sharps handling (secondary)	No cuts, tears, or separation at seams	When cut or torn
Oven mitts	Handling hot items in kitchen	No burns, thinning, or exposed lining	When damaged or protection is reduced
Non-slip footwear	Kitchen duties; wet outdoor surfaces	Adequate tread; no delamination	When tread is worn flat or footwear is damaged

APPENDIX I — CHEMICAL AND BIOLOGICAL EXPOSURE CONTROL PLAN SUMMARY

Full Safety Data Sheets are maintained in the WHMIS binder at each facility. This summary provides a quick-reference guide for the most common exposures.

Hazardous Agent	Exposure Route	Immediate Response	Medical Follow-Up
Bleach / sodium hypochlorite	Skin, eye, inhalation	Skin: wash 20 min with water. Eye: flush 20 min with water. Inhalation: fresh air immediately	If eye contact: ophthalmology assessment. Significant inhalation: emergency
Quaternary ammonium disinfectant	Skin, eye, inhalation	Flush with water 15–20 minutes. Remove contaminated clothing	Medical assessment if irritation persists beyond 30 minutes
Blood / potentially infectious bodily fluid	Skin laceration, mucous membrane, eye splash, needlestick	Wash skin 20 min soap/water; flush eyes/mucous membranes 15 min with water	Emergency assessment within 2 hours for PEP evaluation. Incident Report + WorkSafeBC report if medical treatment required
Sharps (needlestick)	Skin penetration	Wash wound gently with soap and water for 20 min; do not squeeze or suck wound	Emergency assessment within 2 hours; hepatitis B, C, and HIV PEP evaluation
Oven cleaner / drain cleaner (corrosive)	Skin, eye	Immediate flush with large amounts of water; remove contaminated clothing	Emergency if eye contact or burn. Medical assessment for significant skin exposure
Unknown substance (suspected illicit drug — capsule, powder, residue)	Skin, inhalation	Do not touch; ventilate area; call supervisor immediately; gloves before any contact; wash hands thoroughly	Seek medical advice if symptomatic; report to supervisor and MCFD

APPENDIX J — ANNUAL OHS PERFORMANCE DASHBOARD

Reporting Period	_____ to _____
Prepared By	
Date Prepared	
Presented to Board	

Key Performance Indicator	Result	Target / Notes
Total recordable injuries (all injuries requiring medical attention beyond first aid)		Target: year-over-year reduction
Lost-time injuries (injuries resulting in any time away from work)		Target: year-over-year reduction
Lost-time injury frequency rate (LTIFR = $LT I \div \text{hours worked} \times 200,000$)		
Near-miss reports submitted		Target: increase reflects stronger reporting culture
Serious incidents (WorkSafeBC reportable)		Target: 0
Workplace violence incidents (physical)		Target: year-over-year reduction
Workplace violence incidents resulting in lost time		Target: 0
Incident investigations completed within required timeline		Target: 100%
WorkSafeBC reportable incidents notified within required timeframe		Target: 100%
Monthly workplace inspections completed (all facilities)	/ 12 per facility	Target: 100%
Corrective actions from inspections closed within target date		Target: 90%+
JHSC meetings held as scheduled	/ 12	Target: 100%
Mandatory OHS training compliance — all workers		Target: 100%
NVCI certification compliance — direct-care staff		Target: 100%
First Aid certification compliance — designated attendants		Target: 100%
WorkSafeBC claims filed during period		
Average days lost per lost-time injury		
Worker OHS satisfaction survey score (% satisfied or very satisfied)		Target: 70%+

Summary of significant incidents, trends, and recommended priorities for the coming year:

Health and Safety Coordinator Signature / Date	Executive Director Signature / Date

27. FIRE SAFETY AND EMERGENCY EVACUATION PROGRAM

27.1 Legislative Authority

This section is governed by the BC Fire Code (Fire Services Act, RSBC 1996, c. 144), WorkSafeBC OHS Regulation Part 4 (General Conditions) and Part 32 (Emergency Preparedness), and the BC Building Code. MCFD licensing standards also require documented emergency evacuation plans for all residential care facilities.

27.2 Fire Hazard Assessment

Each facility conducts and documents an annual fire hazard assessment covering:

- Identification of all ignition sources (stoves, heating equipment, electrical panels, candles, smoking areas)
- Identification of all fuel sources and their storage locations (cleaning products, paper waste, bedding materials)
- Assessment of means of egress including door widths, emergency lighting, and exit signage
- Review of fire detection, alarm, and suppression systems and their inspection history
- Assessment of occupant-specific evacuation needs (mobility limitations, cognitive challenges, sleep status)

The fire hazard assessment is reviewed following any structural change to the facility, any fire incident, and at minimum annually. Results are recorded on the Fire Hazard Assessment Form (Appendix K) and remediation actions are tracked to completion.

27.3 Facility Fire Safety Equipment Requirements

Equipment	Requirement
Smoke detectors	Installed in every sleeping room, hallway, and common area; tested monthly; battery backup
Carbon monoxide detectors	Required in all facilities with fuel-burning appliances or attached garages; tested monthly
Fire extinguishers	Minimum one ABC extinguisher per floor; kitchen area Class K extinguisher; inspected annually
Emergency lighting	Battery-backed emergency lighting at all exits and stairways; tested monthly
Exit signage	Illuminated exit signs at all required egress points; tested monthly
Fire alarm system	Interconnected system required in multi-unit or multi-storey facilities; inspected annually by certified contractor
Sprinkler system	Where required by BC Building Code; inspected quarterly by certified contractor

27.4 Evacuation Plan Requirements

Every facility maintains a written Evacuation Plan that includes:

- Floor plan(s) of the facility showing: all rooms; all exits (primary and secondary); fire extinguisher locations; fire alarm pull station locations; assembly point location(s)

- Designated roles for each shift: Evacuation Leader (shift lead); Floor Sweeper(s); Assembly Point Monitor; 911 Caller
- Specific procedures for evacuating youth with mobility limitations or who may resist evacuation
- Procedures for nighttime evacuations when youth are sleeping and staff levels may be lower
- Assembly point location(s) at a safe distance from the building
- Procedures for accounting for all occupants at the assembly point using the shift log roster
- Re-entry authorization procedure (fire department permission only)

The Evacuation Plan is posted on every floor of the facility and reviewed with all new staff during orientation.

27.5 Evacuation Drill Requirements and Records

Drill Type	Frequency	Documentation Required
Announced fire evacuation drill	Quarterly (every 3 months)	Date, time, duration, participants, deficiencies noted, corrective actions
Unannounced drill (night or weekend shift)	Once annually	Same as above plus staff names on shift
Full evacuation with all occupants	Twice annually minimum	Include youth names (anonymized in records)
Lockdown / shelter-in-place drill	Annually	Scenario description, deficiencies, corrective actions
Tabletop exercise with all staff	Annually	Scenario, attendees, outcomes

Drill records are retained for a minimum of five (5) years and made available to WorkSafeBC and MCFD licensing officers on request.

27.6 Response to Fire Alarm Activation

Immediate Actions (within the first 60 seconds)

- Evacuate all occupants immediately using the primary exit route; do not stop to investigate the alarm source
- The Evacuation Leader confirms all staff roles are activated
- The Floor Sweeper checks every room on every floor, calling out and opening all doors while escaping
- The 911 Caller contacts emergency services immediately upon evacuation (do not wait for confirmation of fire)
- All occupants proceed directly to the designated assembly point; no one enters or re-enters the building

Assembly Point

- The Evacuation Leader conducts a roll call of all occupants against the shift log roster
- Any missing occupant is reported immediately to the responding fire department; staff do not re-enter the building to search
- The Program Manager or on-call supervisor is notified immediately

After the Incident

- Complete an Incident Report within 24 hours
- Conduct a staff debrief to identify any evacuation gaps
- Notify MCFD licensing office of any fire incident, regardless of severity

27.7 Fire Safety Training

All staff complete fire safety training within the first week of employment and annually thereafter. Training covers: the facility's evacuation plan and their role; operation of fire extinguishers (using the PASS technique: Pull, Aim, Squeeze, Sweep); recognition of fire alarm system components; evacuation of persons requiring assistance; and documentation of drills and incidents.

28. FALL PREVENTION AND PROTECTION PROGRAM

28.1 Legislative Authority

WorkSafeBC OHS Regulation Part 11 (Fall Protection) applies wherever workers are exposed to a risk of falling two (2) metres or more, or into or onto a hazardous surface or equipment at any height. This section also addresses slip, trip, and fall hazards at ground level, which represent the most frequent source of injury in residential care settings.

28.2 Scope of Fall Hazards in Residential Care

Residential care workers face fall hazards in several routine work contexts:

Hazard Context	Examples and WorkSafeBC Regulation Reference
Ground-level slips and trips	Wet floors, cluttered hallways, unsecured rugs, outdoor pathways (OHS Reg. Part 4, s.4.26)
Stairways	Inadequate handrails, poor lighting, carrying items while ascending/descending
Ladders	Changing light bulbs, accessing high shelves, exterior maintenance (OHS Reg. Part 11, s.11.2–11.29)
Working at heights outside	Exterior window cleaning, gutter clearing, grounds maintenance (OHS Reg. Part 11)
Working on roofs	Only authorized personnel; fall protection required (OHS Reg. Part 11, s.11.2)
Assisting youth with mobility	Supporting youth during transfers or physical activities that may cause caregiver instability

28.3 Hierarchy of Fall Controls

Before authorizing any work at heights, the supervisor applies the hierarchy of controls:

- Elimination: re-design the task to eliminate the fall hazard (e.g., relocate stored items to accessible shelves)
- Engineering controls: install fixed guardrails, handrails, stair nosing, and anti-slip surfaces
- Administrative controls: restrict access to elevated areas; require two-person teams; schedule work when youth are in school
- PPE: use properly rated ladders, safety harnesses, and non-slip footwear as the last line of defence

28.4 Ladder Safety Requirements

When ladder use cannot be eliminated, the following requirements apply under OHS Regulation Part 11:

- Only ladders rated for the expected load (worker weight plus tools/materials) are used; damaged or defective ladders are removed from service immediately and tagged “Do Not Use”
- Ladders are inspected before each use for: cracked or broken rungs; damaged feet or caps; bent or twisted side rails; loose hardware
- Step ladders are used only in the fully open, locked position on a firm, level surface

- Extension ladders are placed at a 4:1 ratio (for every 4 feet of height, the base is 1 foot from the wall) and extend at least 1 metre above the landing point
- Workers maintain three points of contact at all times while on a ladder
- Tools and materials are raised or lowered using a tool bag or rope, never carried by hand while climbing
- No person stands on the top two rungs of a step ladder or the top three rungs of an extension ladder
- Ladders are never placed in front of a doorway without a spotter or door locked

28.5 Ground-Level Slip and Trip Prevention

- All interior walkways, stairways, and exits are kept clear of clutter at all times
- Wet floors are immediately blocked with signage and dried as quickly as possible
- Non-slip mats are maintained at all exterior entry points and in bathrooms used by youth
- Stairways have functioning handrails on both sides and adequate lighting at all times
- Extension cords and charging cables are secured and routed away from walkways
- Outdoor pathways are cleared of ice and snow before the start of each shift in winter

28.6 Working at Heights (2 Metres or More)

Any work at heights of 2 metres or more requires a written fall protection plan under OHS Regulation s.11.2 before work begins. The plan must identify: the specific fall hazards; the fall protection system to be used (guardrails, safety net, or personal fall arrest system); rescue procedures if a worker is suspended following a fall. The fall protection plan is reviewed and signed by the Program Manager before work commences.

28.7 Personal Fall Arrest Equipment

Where fall arrest equipment is required, only CSA-certified equipment is used. Workers inspect harnesses, lanyards, and anchor points before each use. Any equipment subjected to a fall arrest load is immediately removed from service, inspected by the manufacturer or certified inspector, and replaced unless certified as safe. Equipment is stored away from UV light, chemicals, and heat when not in use.

29. NOISE AND VIBRATION EXPOSURE PROGRAM

29.1 Legislative Authority

WorkSafeBC OHS Regulation Part 7 (Noise, Vibration, Radiation, and Temperature) governs occupational exposure to noise and vibration. Section 7.1 to 7.29 establish permissible exposure limits (PELs) and control requirements. While severe noise hazards are less common in residential care than in industrial settings, specific tasks and environments create significant noise exposure that must be managed.

29.2 Noise Hazards in Residential Care

Noise Source	Approximate dBA Level / Risk
Normal office environment	50–60 dBA (safe)
Busy residential care home, multiple youth active	65–75 dBA (monitor)
Youth in acute behavioural crisis, screaming	80–100 dBA (hazardous with prolonged exposure)
Lawn mowers, power tools	85–95 dBA (hearing protection required)
Vacuum cleaners used for extended periods	70–80 dBA (monitor exposure duration)
Vehicle traffic (extended transport duties)	70–85 dBA (consider duration)

29.3 Permissible Exposure Limits

Under OHS Regulation s.7.2, the daily noise exposure limit (LEQ) is 85 dBA (A-weighted). For every 3 dBA increase above 85, the permissible exposure time is halved (the exchange rate):

Noise Level (dBA)	Maximum Daily Exposure Duration
85 dBA	8 hours
88 dBA	4 hours
91 dBA	2 hours
94 dBA	1 hour
97 dBA	30 minutes
100 dBA or above	15 minutes or less

29.4 Noise Controls

- Administrative controls: rotate staff assigned to high-noise tasks; limit continuous exposure to high-noise environments; schedule loud maintenance activities when youth are at school
- Engineering controls: ensure facility mechanical equipment (HVAC, appliances) is maintained in good working order; address excessive noise from equipment promptly

- PPE: foam or reusable earplugs (NRR 25+) are available in the facility and mandatory for workers using loud power tools or equipment

29.5 Hearing Conservation

Workers who report ringing in their ears (tinnitus), difficulty hearing conversations, or muffled hearing following work shifts must report this to their supervisor immediately. The supervisor refers the worker to occupational health assessment and documents the report. WorkSafeBC must be notified if occupational hearing loss is identified.

29.6 Hand-Arm Vibration (HAVS)

Workers who use vibrating tools (power washers, electric screwdrivers, hedge trimmers) for extended periods are at risk of Hand-Arm Vibration Syndrome. Symptoms include tingling, numbness, or whitening of fingers (Raynaud's phenomenon). Workers must report these symptoms immediately. Exposure is managed by limiting continuous tool use to thirty (30) minute intervals with rest breaks.

30. CONFINED SPACE ENTRY PROGRAM

30.1 Legislative Authority

WorkSafeBC OHS Regulation Part 9 (Confined Spaces) applies to any confined space that workers may enter. A confined space is defined as an enclosed or partially enclosed space: not designed for continuous human occupancy; having restricted means of entry or exit; and which may have or develop a hazardous atmosphere, engulfment risk, entrapment risk, or other serious physical hazard.

30.2 Confined Spaces in Residential Care Facilities

Potential confined spaces at residential care facilities include:

- Crawl spaces beneath buildings
- Attic spaces with restricted access
- Utility vaults or underground service access points
- Large storage tanks (oil, water, chemical)
- Septic tanks or holding tanks (where applicable)
- Electrical vaults

30.3 Permit-Required Confined Space Entry

Entry into any permit-required confined space requires:

- A written Confined Space Entry Permit completed before entry, signed by the Program Manager or a designated competent supervisor
- Atmospheric testing for oxygen level (19.5%–23.5% required), combustible gases (less than 10% of LEL), and toxic gases (H₂S, CO) before and during entry
- A trained attendant stationed outside the confined space for the entire duration of entry, maintaining communication with the entrant
- A rescue plan and rescue equipment readily available before entry begins; emergency services notified where required
- Lockout/tagout of all energy sources serving the confined space before entry

No worker enters a confined space without completion of all permit requirements. Workers who discover an unplanned need to enter a confined space must stop work and contact their supervisor before proceeding.

30.4 Training

Workers who may be required to enter confined spaces, act as attendants, or participate in confined space rescue complete WorkSafeBC-approved confined space entry training before carrying out those duties. Training records are maintained in each worker's personnel file.

30.5 Non-Entry Procedures

Where confined space entry can be avoided through remote inspection tools (cameras, mirrors, extended-reach instruments), non-entry procedures are preferred and used wherever safely possible.

31. LOCKOUT AND TAGOUT (LOTO) ENERGY CONTROL PROGRAM

31.1 Legislative Authority

WorkSafeBC OHS Regulation Part 10 (De-Energization and Lockout) requires that energy sources be controlled before workers perform maintenance, servicing, or repair on equipment. This prevents unexpected energization, startup, or release of stored energy that could injure workers.

31.2 Application in Residential Care

LOTO procedures apply in residential care facilities for maintenance and repair tasks including:

- Electrical repairs (circuit breaker work, appliance servicing, light fixture replacement)
- HVAC system maintenance and filter replacement on powered systems
- Plumbing work involving pressurized water or gas lines
- Appliance repair or disconnection
- Any work inside or adjacent to electrical panels

31.3 Lockout Procedure

The sequence for locking out energy sources before beginning work:

Step	Action Required
1. Identify	Identify all energy sources serving the equipment (electrical, pneumatic, hydraulic, gravity, thermal)
2. Notify	Notify all affected workers that equipment is being locked out
3. Shutdown	Shut down the equipment using normal stopping procedures
4. Isolate	Isolate the energy source at the control point (breaker, valve, disconnect)
5. Lock	Apply a personal lockout device; each worker applies their own lock
6. Tag	Attach a “Danger — Do Not Operate” tag to the lockout device
7. Release stored energy	Release, restrain, or otherwise render safe any stored or residual energy
8. Verify isolation	Attempt to start the equipment; verify with test instruments that energy is absent

31.4 Lockout Equipment

The facility maintains a lockout kit containing: individually keyed padlocks (one per staff member authorized to perform lockout); lockout hasps; lockout tags; circuit breaker lockout devices; and valve lockout devices. Lockout equipment is stored in a designated, accessible location and is never used for any purpose other than energy isolation.

31.5 Contractor Lockout

When an external contractor performs maintenance requiring lockout, the contractor’s lockout procedure is reviewed against organizational requirements. The Program Manager or designated supervisor ensures all energy sources are identified to the contractor and that the contractor’s locks are in place before work begins. Organizational staff must not remove a contractor’s lock, and contractors must not remove organizational locks.

32. ELECTRICAL SAFETY PROGRAM

32.1 Legislative Authority

WorkSafeBC OHS Regulation Part 19 (Electrical Safety) and the BC Safety Standards Act (Electrical Safety Regulation) govern electrical work and equipment in BC workplaces. Residential care facilities contain numerous electrical hazards that require ongoing management.

32.2 General Electrical Safety Requirements

- Only qualified electricians (BC Certificate of Qualification) perform electrical work on facility wiring, panels, or permanently installed equipment
- All electrical work requiring a permit under the BC Electrical Code is permitted and inspected by a Safety Authority inspector before energization
- Ground fault circuit interrupter (GFCI) protection is required for all outlets within 1.5 metres of water sources (kitchens, bathrooms, laundry areas)
- Extension cords are a temporary measure only; permanent extension cord installations are prohibited; damaged or cracked cords are removed from service immediately
- Multi-outlet adapters (“power bars”) must be CSA-certified and include overload protection; daisy-chaining of power bars is prohibited
- Electrical panels must be accessible and clear of stored items at all times; panel directories are current and legible

32.3 Electrical Hazard Inspection

The shift lead inspects the facility monthly for electrical hazards as part of the standard workplace inspection (Section 20). Electrical hazards identified include:

- Damaged, frayed, or cracked electrical cords or plugs
- Outlets or switches with damaged covers or visible damage to the face
- Overloaded circuits (frequently tripping breakers)
- Signs of overheating: scorch marks, melted plastic, burning smell near electrical equipment
- Equipment that sparks, shocks, or behaves abnormally

Any electrical hazard is tagged, the affected outlet or equipment is taken out of service immediately, and repair by a qualified electrician is arranged. Workers must not attempt to repair electrical equipment themselves.

32.4 Youth Safety around Electricity

Electrical safety for youth in the facility includes:

- Outlet covers installed on all unused outlets accessible to young children
- Electrical appliances not in use are unplugged and stored when youth are present if the appliance presents a hazard (toasters, blenders, hair dryers)
- Youth are never permitted to use electrical appliances unsupervised until assessed as competent; this is documented in the care plan
- Youth are educated age-appropriately about electrical safety as a life skills topic

33. ENVIRONMENTAL AND INDOOR AIR QUALITY PROGRAM

33.1 Legislative Authority

WorkSafeBC OHS Regulation Part 4 (General Conditions, ss. 4.54–4.80), Part 5 (Chemical Agents and Biological Agents), and Part 6 (Substance-Specific Requirements) address environmental workplace hazards. Indoor air quality (IAQ) is particularly important in residential care settings where people live and work continuously in the same space.

33.2 Indoor Air Quality

Sources of indoor air quality concerns in residential care facilities include:

IAQ Hazard	Control Measures
Mould and moisture	Monthly inspection of bathrooms, laundry, windows, and roof areas; address leaks within 48 hours; clean visible mould with appropriate PPE; report extensive mould to Program Manager immediately
Cleaning product fumes	Use cleaning products in well-ventilated areas; follow SDS dilution ratios; avoid mixing bleach with ammonia or acids; store in original labeled containers
Carbon monoxide (CO)	CO detectors on every level; annual inspection of gas appliances and furnace by certified technician; any CO alarm treated as a real emergency
Biological contaminants (mould, bacteria)	HVAC filters changed quarterly; humidifier water changed and units cleaned weekly; refrigerator coils cleaned annually
Second-hand smoke	Smoking prohibited inside all facilities and within 3 metres of any exterior entrance or air intake, per BC Tobacco and Vapour Products Control Act
Poor ventilation	HVAC systems maintained per manufacturer schedule; air circulation assessed if staff or youth report persistent headaches or respiratory symptoms

33.3 Asbestos

Residential care facilities constructed before 1990 may contain asbestos-containing materials (ACM) in insulation, floor tiles, ceiling tiles, or pipe wrap. Under OHS Regulation Part 6 (Asbestos), any demolition, renovation, or disturbance of materials in these facilities requires a building assessment by an accredited asbestos assessor before work begins. Workers must not disturb any suspect material without a prior assessment. If asbestos is identified, abatement is conducted only by WorkSafeBC-registered asbestos abatement contractors. The Program Manager maintains records of any asbestos assessments and the location and condition of known ACM in each facility.

33.4 Lead

Facilities built before 1978 may have lead-based paint on interior or exterior surfaces. Disturbance of lead paint during renovation creates a serious inhalation hazard. Before any sanding, scraping, or removal of painted surfaces in pre-1978 buildings, a lead risk assessment is conducted. Lead abatement work is performed by workers trained in lead handling, using appropriate respiratory protection (minimum P100 respirator) and disposal procedures for lead-contaminated waste.

33.5 Pest Control

Pest infestations (rodents, cockroaches, bedbugs) create biological hazards for workers and occupants. The Program Manager is notified immediately of any evidence of pests. Pest control is conducted by a licensed pest control operator. Staff who perform any role in pest control measures (placing traps, applying pesticide) must be informed of the products used via SDS and provided appropriate PPE. Youth are relocated to unaffected areas of the facility or temporarily to another site during pesticide applications requiring vacancy.

33.6 Legionella Prevention

Hot water systems in residential care facilities can harbour Legionella bacteria if water temperatures fall below safe thresholds. Hot water heaters are set at a minimum of 60°C (140°F) at the heater to prevent Legionella growth, with anti-scald mixing valves at point of use to prevent burns. The Program Manager ensures hot water systems are inspected and flushed annually by a qualified plumber.

34. EXPANDED WORKPLACE INSPECTION PROCEDURES AND CHECKLISTS

34.1 Purpose

This section supplements Section 20 (Workplace Inspections) with detailed inspection procedures, frequency requirements, and checklists aligned with all WorkSafeBC OHS Regulation hazard categories applicable to residential care facilities.

34.2 Inspection Types and Frequencies

Inspection Type	Frequency	Conducted By
Shift safety walkthrough	Every shift (brief visual)	Shift lead
Monthly comprehensive inspection	Monthly	Shift lead + one other staff or JHSC member
Quarterly JHSC inspection	Quarterly	JHSC representative(s)
Annual program audit	Annually	Program Manager + external OHS consultant
Post-incident inspection	Within 24 hours of any incident	Supervisor + JHSC member
Pre-occupancy inspection (new or renovated spaces)	Before first occupancy	Program Manager + fire authority

34.3 Comprehensive Monthly Inspection Checklist

Section A: Interior Housekeeping and General Conditions

- All corridors, exits, and stairways clear of clutter and obstructions
- All exit signs illuminated and legible
- Emergency lighting operational (tested)
- Fire extinguishers in place, gauge in green zone, inspection tag current
- Smoke detectors in all sleeping rooms and common areas (test at least one per visit)
- Carbon monoxide detectors present and operational
- No slip or trip hazards observed (loose mats, wet floors, extension cords across walkways)
- No holes, damaged flooring, or broken stair nosing
- All handrails on stairs secure and continuous

Section B: Kitchen and Food Safety

- Refrigerator temperature at or below 4°C (check and record)
- Freezer temperature at or below -18°C (check and record)
- No expired food products in use
- Cleaning chemicals stored separately from food
- Exhaust fan functional and filters clean

- Kitchen Class K fire extinguisher present and current
- No grease buildup on stove or around exhaust

Section C: Electrical

- All outlet and switch covers intact; no exposed wiring
- GFCI outlets operational in kitchen and bathrooms (test reset button)
- Electrical panel accessible, directory current and legible
- No damaged cords observed
- No evidence of overheating (scorch marks, burning smell) at any outlet or appliance

Section D: First Aid

- First aid kit fully stocked (complete contents list attached)
- Eyewash station stocked and within reach of chemical storage areas
- First aid certificate(s) current for on-shift staff
- First aid log available and in use
- AED (if present) service light green; pads within expiry date

Section E: Chemical and WHMIS

- All chemical containers labeled (original label or WHMIS workplace label)
- Safety Data Sheets available and current for all chemicals in use
- No unlabeled or unknown containers present
- Chemicals stored in appropriate, ventilated, locked area
- Quantities on hand consistent with operational need; no excessive stockpiling

Section F: Outdoor Areas and Vehicles

- Exterior walkways clear, no ice or snow accumulation
- Outdoor lighting functional at all entry and egress points
- Fencing and gates secure where required for youth safety
- Vehicle(s) inspected: current insurance, decal, working lights, emergency equipment on board
- No environmental spills, fuel, or chemical storage outdoors without appropriate secondary containment

Section G: Biological Hazard Controls

- Sharps containers present where needed (first aid area, medication storage)
- PPE for blood or body fluid exposure available and accessible
- Biohazard waste disposal procedures posted and being followed
- No evidence of pest activity (rodents, insects, bedbug signs)
- No evidence of mould or water intrusion on ceilings, walls, or in bathrooms

34.4 Inspection Reporting and Follow-Up

Every inspection produces a written report. Hazards identified are classified as:

Priority Level	Criteria	Required Response Time
Priority 1 — Imminent Danger	Hazard presents immediate risk of serious injury or death	Correct immediately; stop work if necessary; report to WorkSafeBC if required by OHS Reg. s.3.2
Priority 2 — Serious Hazard	Hazard could cause injury; risk is real and foreseeable	Correct within 48 hours; interim controls applied

		immediately
Priority 3 — Low Hazard	Nuisance or administrative concern; low immediate risk	Correct within 14 days; include in next JHSC report

All inspection reports are retained for a minimum of three (3) years. Outstanding corrective actions are tracked in the JHSC meeting minutes until closed.

35. HEAT STRESS AND THERMAL WORK ENVIRONMENT PROGRAM

35.1 Legislative Authority

WorkSafeBC OHS Regulation Part 7 (ss. 7.27–7.29) requires employers to assess and control risks from working in hot environments. This program applies to residential care staff during summer heat events, outdoor activities, vehicle transport, and work in non-air-conditioned spaces.

35.2 Heat Stress Risk Factors

The following factors increase the risk of heat illness in residential care work contexts:

- High ambient temperature or humidity, especially during BC heat dome events (temperatures above 35°C)
- Physical exertion during outdoor activities, community outings, or physical intervention with youth
- Vehicle transport without air conditioning or with malfunctioning air conditioning
- Wearing PPE (gloves, face shields) that reduces body heat dissipation
- Staff who are unacclimatized to heat (returning from time off, new staff in summer)
- Staff taking certain medications that reduce heat tolerance (antihistamines, diuretics, some antipsychotics)

35.3 Types of Heat Illness and Recognition

Condition	Symptoms	Response
Heat cramps	Muscle cramping in legs, arms, or abdomen; heavy sweating	Rest in cool area; hydrate with water or electrolyte drink; monitor
Heat exhaustion	Heavy sweating; cool, pale, clammy skin; fast, weak pulse; nausea; dizziness; headache	Move to cool area; loosen clothing; apply cool wet cloths; sip cool water; if vomiting, call 911
Heat stroke	High body temperature (39°C+); hot, red, dry OR damp skin; rapid strong pulse; confusion; possible unconsciousness	Call 911 immediately; cool the worker rapidly; this is a life-threatening emergency

35.4 Heat Stress Controls

Engineering Controls

- Air conditioning or adequate mechanical ventilation in all occupied residential spaces
- Portable fans and cooling devices available for use during heat events
- Sun shading on south and west-facing windows during summer months

Administrative Controls

- During BC Heat Alert / Heat Warning events, shift leads monitor weather and activate the Heat Response Plan
- Community outings scheduled in the morning or late afternoon; avoided during peak heat (12:00–16:00) when temperatures exceed 30°C
- Acclimatization: new staff and those returning after 2+ weeks off are given a 4–5 day gradual acclimatization period when working in heat

- Work-rest ratios applied during heat events: for light work at 35°C, maximum 45 minutes continuous work with 15 minutes rest in cool area

Hydration

- Cool drinking water is available at no cost to all staff at all times; at minimum 250 ml every 20 minutes during heat exposure
- Caffeinated beverages, alcohol, and high-sugar drinks are not a substitute for water

35.5 Cold Stress

Workers performing outdoor tasks in winter (escorting youth to vehicles, shovelling, outdoor activities) are at risk of cold stress. Controls include:

- Appropriate layered clothing; waterproof outer layer; insulated footwear
- Limitation of unprotected outdoor exposure to 30 minutes at temperatures below -10°C windchill
- Recognition of frostbite (skin numbness, whitening) and hypothermia (shivering, confusion, slurred speech); call 911 for hypothermia
- Warm break area available at all times during cold outdoor work

36. INFECTIOUS DISEASE AND BIOLOGICAL HAZARD PREVENTION PROGRAM

36.1 Legislative Authority

WorkSafeBC OHS Regulation Part 6 (Biological Agents, ss. 6.34–6.55) requires employers to assess and control the risk of exposure to biological agents including bacteria, viruses, fungi, and parasites. Residential care workers are at elevated risk of exposure to blood-borne pathogens, respiratory illnesses, and other communicable diseases through close contact with children and youth in care.

36.2 Blood-Borne Pathogen Exposure Prevention

Exposure Scenarios

Residential care workers may be exposed to blood-borne pathogens (HIV, Hepatitis B, Hepatitis C) through:

- Direct contact with blood during first aid, wound care, or physical intervention
- Needlestick injury from discarded syringes found in the facility or on community outings
- Splash of blood or body fluids to eyes, nose, or mouth
- Contact with blood through cuts or open skin wounds on the worker

Standard Precautions

Standard Precautions apply to all blood, bodily fluids (excluding sweat), non-intact skin, and mucous membranes, regardless of whether the source is known to be infectious:

- Disposable nitrile gloves worn for all contact with blood or body fluids
- Wash hands with soap and water immediately after removing gloves; alcohol-based hand sanitizer used when hand-washing is not immediately available
- Face shields or goggles worn when splash risk exists (wound irrigation, managing an actively bleeding youth)
- Sharps (needles, broken glass) are never picked up by hand; use tongs, a broom, or cardboard; dispose in a sharps container
- Contaminated surfaces are cleaned and disinfected with an approved disinfectant (1:10 bleach solution or equivalent)

36.3 Post-Exposure Protocol

In the event of a blood-borne pathogen exposure (needlestick, splash to mucous membrane, bite that breaks skin):

Step	Required Action
1. Immediate first aid	Wash wound with soap and water for 15 minutes; flush eyes with clean water for 15 minutes; do not squeeze or suction the wound
2. Report	Report immediately to the supervisor; do not delay
3. Medical assessment	Seek immediate medical evaluation (emergency department or occupational health); bring the WorkSafeBC Form 7 and source information if known
4. WorkSafeBC report	Supervisor completes WorkSafeBC employer report (Form 7) within 72 hours
5. Follow-up	Worker follows prescribed medical follow-up schedule; source testing arranged where possible and consented

36.4 Respiratory Illness Prevention

Residential care workers and youth are in close contact, increasing transmission risk for respiratory illnesses including influenza, respiratory syncytial virus (RSV), COVID-19, and tuberculosis. Controls include:

- Annual influenza vaccination strongly encouraged for all staff; WorkSafeBC considers influenza occupational if contracted in care settings
- N95 or equivalent respirators (fit-tested annually) available for staff providing care to youth with confirmed or suspected airborne communicable illness
- Surgical masks available for use during close-contact activities when symptomatic youth cannot be isolated
- Staff who are symptomatic with fever plus respiratory symptoms must not report to work; sick leave is available without penalty
- Enhanced surface cleaning of high-touch areas (door handles, light switches, shared devices) during any respiratory illness outbreak

36.5 MRSA, VRE, and Resistant Organisms

Some children and youth in residential care may carry antibiotic-resistant organisms (MRSA, VRE). Workers are informed (on a need-to-know basis consistent with youth confidentiality requirements) when Contact Precautions are required. Contact Precautions require:

- Gloves and gown before entering the youth's room or having direct contact
- Hand hygiene before and after contact
- Dedicated or single-use care items for the youth
- Consultation with MCFD and the youth's prescriber regarding isolation requirements

36.6 Sharps Found in the Facility

Found sharps (needles, syringes) are treated as potentially contaminated blood-borne pathogen hazards:

- Never pick up by hand; use disposable tongs, cardboard, or a sharps pickup device
- Place directly into a puncture-resistant sharps container; do not recap, bend, or break needles
- Remove the sharps container for disposal when three-quarters full; contact the facility manager for disposal
- Document the find in the shift log: location, time, how disposed of
- If a youth is found in possession of a syringe, notify the supervisor immediately; do not attempt to take it from the youth by force

37. PSYCHOLOGICAL HEALTH AND SAFETY PROGRAM (WorkSafeBC Standards)

37.1 Legislative Authority and Standards

WorkSafeBC's policies under the Workers Compensation Act (WCA) recognize mental disorder claims arising from workplace events or cumulative occupational exposures. Bill 23 (2022 amendment to the WCA) extended presumptive coverage for post-traumatic stress disorder (PTSD) and acute stress disorder to all workers, not just first responders. The National Standard of Canada for Psychological Health and Safety in the Workplace (CSA Z1003-13/BNQ 9700-803) provides the framework for this program.

37.2 Thirteen Psychosocial Factors (CSA Z1003)

The CSA National Standard identifies thirteen workplace factors that influence psychological health and safety. Evergreen Community Care's program addresses all thirteen:

Psychosocial Factor	Relevant Organizational Practice at Evergreen
Psychological support	EFAP access; reflective supervision; non-punitive incident reporting
Organizational culture	Open-door policy; harassment prevention; respectful workplace policy
Clear leadership and expectations	Documented job descriptions; regular supervision meetings; clear chain of command
Civility and respect	Code of conduct; anti-harassment policy; peer review of workplace conflicts
Psychological competencies of leaders	Supervisor training includes trauma-informed and psychologically safe leadership
Growth and development	Professional development fund; succession planning; training matrix
Recognition and reward	Performance recognition program; service anniversary acknowledgement
Involvement and influence	Staff participation in policy review; JHSC membership; team meetings
Workload management	Scheduling policy; fatigue management; overtime limits; adequate staffing ratios
Engagement	Mission-aligned hiring; team-building activities; annual all-staff survey
Balance	Reasonable scheduling; vacation use encouraged; EAP counselling for personal stressors
Psychological protection (from harassment)	Zero-tolerance harassment policy; confidential complaint process; no retaliation

37.3 Secondary Traumatic Stress and Compassion Fatigue

Residential care workers who provide care to traumatized youth are at significant risk of secondary traumatic stress (STS) and compassion fatigue. These are recognized occupational injuries under the WCA. Signs include:

- Intrusive thoughts or dreams related to youth's trauma disclosures
- Emotional numbing or detachment from work or personal relationships
- Hypervigilance, irritability, or heightened anxiety outside of work

- Decline in job satisfaction; cynicism about outcomes
- Physical symptoms: sleep disturbance, chronic fatigue, appetite changes

Organizational Supports

- Reflective supervision provided individually or in groups at least bi-weekly; this is a scheduled, protected part of the supervisor's role
- Critical Incident Stress Debriefs (CISD) offered within 72 hours of any Critical Incident
- EFAP provides up to eight (8) free confidential counselling sessions per year; referrals made without requiring disclosure of the reason to the employer
- Peer support network: trained peer supporters available to provide informal support
- Annual team wellness check-in to assess workload, morale, and team cohesion

37.4 PTSD and Acute Stress Disorder: Workplace Injury Claims

If a worker experiences symptoms of PTSD or acute stress disorder following a workplace incident or cumulative exposure, this is a WorkSafeBC compensable injury. Workers are encouraged to report and claim without stigma. Supervisors are trained to respond supportively and to avoid actions that could discourage a legitimate claim. The organization never discourages or impedes a worker's right to report and claim for a psychological injury.

37.5 Psychologically Safe Incident Reporting

All incident reporting, including near-miss and psychological harm reports, is conducted in a psychologically safe environment. No worker faces discipline, adverse job action, or ostracism for making a genuine safety report. Workers who observe a psychologically unsafe response to an incident report notify the Program Manager or the JHSC worker representative immediately.

38. SAFE CLIENT HANDLING AND MUSCULOSKELETAL INJURY PREVENTION — RESIDENTIAL CARE SPECIFIC

38.1 Legislative Authority

WorkSafeBC OHS Regulation Part 4 (s. 4.46–4.53) and the Health Care and Residential Facilities Regulation require employers to assess and control musculoskeletal injury (MSI) hazards. In residential care settings, MSI hazards arise from manual handling of objects, assisting youth with physical needs, and physical intervention.

38.2 Residential Care-Specific MSI Hazards

Task	MSI Hazard
Assisting a youth with mobility (transfers, lifting)	Back, shoulder, and knee strain; awkward postures
Physical intervention / de-escalation holds	Sudden force; twisting; impact; back and shoulder strain
Carrying laundry, groceries, or equipment	Overexertion; back and shoulder strain from load weight or awkward carry position
Extended computer documentation at end of shift	Repetitive strain; neck and shoulder tension from poor workstation setup
Driving (extended shift transport duties)	Seat posture; vibration; neck and back tension from long periods in vehicle
Housekeeping tasks (scrubbing, mopping, vacuuming)	Repetitive motion; bent or twisted postures

38.3 Youth Transfer and Mobility Assistance

Before assisting any youth with mobility needs, a mobility assessment and safe handling plan is documented in the youth's care plan. The plan specifies:

- The type and level of assistance required
- Required equipment (transfer belt, slide board, mechanical lift if applicable)
- Number of staff required for the transfer
- Techniques to be used and techniques that must not be used

No worker assists a youth with a transfer requiring mechanical lift without appropriate equipment. Two-worker transfers are required for any transfer assessed as high risk. Workers report any change in a youth's mobility that affects transfer safety to the supervisor immediately.

38.4 Physical Intervention MSI Prevention

Physical intervention (Non-Violent Crisis Intervention or equivalent) creates acute MSI risk due to sudden, unplanned forces. Controls include:

- All staff are trained and annually re-certified in the organization's approved physical intervention approach
- Physical intervention is used as a last resort after all de-escalation strategies are exhausted
- Workers report any musculoskeletal symptom following physical intervention immediately; early reporting enables early treatment

- Physical intervention techniques that create high injury risk (full floor restraints, wrist-lock techniques not covered in approved training) are not used

38.5 Safe Lifting Principles

For all manual handling tasks, workers apply safe lifting principles:

- Assess the load before lifting: weight, size, shape, centre of gravity
- Plan the lift: clear the path; identify the destination; determine if assistance or equipment is needed
- Position close to the load; feet shoulder-width apart; stable base
- Bend at the knees, not the waist; keep the back straight
- Lift smoothly using leg muscles; hold the load close to the body
- Never twist the torso while lifting; turn the entire body with feet
- Set down using the same technique as lifting

38.6 Workstation Ergonomics

Documentation workstations are assessed ergonomically and adjusted for each user where possible:

- Monitor at arm's length; top of screen at eye level
- Keyboard and mouse at elbow height; wrists straight
- Chair adjusted so feet are flat on the floor; lumbar support in use
- Frequent short breaks (at least 5 minutes every hour) from computer work
- Workers reporting persistent neck, shoulder, wrist, or back discomfort from computer use are referred for a formal ergonomic assessment

39. MANAGING YOUTH BEHAVIOUR: WORKER SAFETY PROCEDURES

39.1 Purpose

Children and youth in residential care may exhibit challenging behaviours including verbal aggression, property destruction, self-harm, or physical aggression toward staff. This section provides practical safe work procedures for managing these situations in a way that protects worker safety while maintaining a trauma-informed, therapeutic approach.

39.2 Behaviour Risk Assessment

Before a worker is assigned to work with a specific youth, they review the youth's:

- Behaviour Support Plan (BSP) including identified triggers, escalation patterns, and de-escalation strategies
- History of physical aggression toward staff, including type and frequency of incidents
- Current medications and their potential behavioural effects
- Known trauma triggers and environmental factors that increase risk

Newly placed youth without a completed BSP are treated as high-risk until baseline assessment is completed. The supervisor briefs new staff on each youth's profile at every shift handover.

39.3 Environmental Safety Controls

- Hazardous items (knives, scissors, sharp tools) are stored in locked locations accessible only to staff
- Medication is locked at all times except during active administration
- Ligature risks are assessed in bedrooms and bathrooms and mitigated as identified in each youth's care plan
- Staff movement within the facility is planned to ensure they are never cornered or isolated with a youth who has a history of physical aggression; escape routes are always maintained
- Panic alarm or personal duress alarm devices are available in high-risk programs

39.4 De-Escalation as the Primary Intervention

Physical intervention is always the last resort. De-escalation strategies are the primary tool and are used in sequence:

- Early recognition: identify escalation cues (pacing, verbal repetition, increased respiration, flushed appearance) and intervene before crisis
- Create space: give the youth physical and emotional space; reduce demands and stimulation
- Calm presence: use a slow, low, calm voice; avoid confrontational body language
- Validate and empathize: acknowledge the youth's feelings without agreeing with unsafe behaviour
- Offer choices: give the youth a sense of control with limited, realistic options
- Utilize the BSP: implement the specific strategies identified in the youth's plan

39.5 When Physical Intervention Is Required

If a youth poses an immediate risk of serious bodily harm to themselves or others and all other interventions have been exhausted:

- Use only the techniques covered in the organization's approved physical intervention training (e.g., Non-Violent Crisis Intervention)
- Two staff members are involved in any planned physical intervention where staffing allows; lone workers call for backup before intervening unless delay would result in immediate injury

- Use the minimum force necessary for the minimum duration necessary; release as soon as it is safe to do so
- Monitor the youth's breathing and responsiveness throughout; release immediately if respiratory distress is observed
- Call 911 if the intervention cannot be safely managed with available staff
- Document every physical intervention in detail within 2 hours; report to the supervisor and to MCFD as required

39.6 Post-Incident Responsibilities

- All involved staff are assessed for injury immediately after the incident
- Staff are offered EFAP referral and peer support before the end of the shift
- A full incident investigation is completed within 24 hours (Section 11)
- Physical intervention incidents are reviewed at the next team meeting for learning; trends are reported quarterly to the JHSC
- Workers' compensation claims for injuries sustained during physical intervention are supported and expedited by the supervisor

40. WorkSafeBC EMPLOYER OBLIGATIONS: COMPREHENSIVE REFERENCE

40.1 Purpose

This section consolidates Evergreen Community Care's key obligations under the Workers Compensation Act (WCA) and WorkSafeBC OHS Regulation, organized by topic for quick supervisor and JHSC reference.

40.2 Reporting Obligations

Obligation	Requirement and Timeline
Serious injury or death	Immediately notify WorkSafeBC at 1-888-621-7233; do not disturb the scene except to help the injured worker or prevent further injury (WCA s. 172)
Serious incident (potential for serious injury)	Notify WorkSafeBC immediately when specified serious incidents occur (WCA s. 172)
Employer Injury Report (Form 7)	Within 3 business days of learning of any worker injury requiring medical treatment or lost time (WCA s. 150)
Accident scene preservation	Maintain the scene undisturbed until investigated and cleared by WorkSafeBC or 24 hours, whichever comes first, for serious incidents (WCA s. 173)
Occupational disease	Report as soon as practicable after diagnosis; Form 7 required (WCA s. 150)
Worker psychological injury (PTSD, acute stress disorder)	Presumptive coverage; employer reports same as physical injury; does not require proof of specific incident

40.3 Posting and Written Program Obligations

The following documents must be posted or readily available in each facility:

Required Document	Location / Requirement
WorkSafeBC "In Case of Injury at Work" poster	Posted in a prominent location accessible to all workers (WCA s. 186)
This OHS Program	Available to all workers; workers informed of its location at orientation
Safe Work Procedures	Accessible at the point of work; posted in relevant areas (OHS Reg. s.4.3)
First aid facilities notice	Posted near first aid station
JHSC meeting minutes	Posted for worker review within one week of each JHSC meeting
WorkSafeBC inspection orders	Posted at the workplace and complied with promptly (WCA s. 197)
Hazardous product labels and SDS	Labels on all containers; SDS immediately accessible during all work shifts (WHMIS 2015)

40.4 Training Obligations

WorkSafeBC requires that workers receive specific training before carrying out work involving significant hazards:

Training	Regulatory Requirement
New worker orientation	Before the worker begins work (OHS Reg. s.3.23)
Young worker orientation (under 25)	Enhanced orientation required (OHS Reg. s.3.23(2))
WHMIS 2015	Before working with or near hazardous products (Hazardous Products Act)
First aid	Based on facility classification; maintain current certificate (OHS Reg. Part 3)
Emergency procedures	Before working in the facility; annually refreshed (OHS Reg. s.4.13)
Specific hazard training	Before assigning worker to task with identified hazard (OHS Reg. s.3.23(3))

40.5 Right to Refuse Unsafe Work

OHS Regulation s.3.12 gives workers the right to refuse work that they believe presents an undue hazard. The refusal process:

- Worker reports the concern to their supervisor immediately
- Supervisor investigates immediately and either eliminates the hazard or confirms the work is safe
- If the matter is not resolved, a JHSC member is involved; the worker may remain in a safe area during investigation
- If still unresolved, a WorkSafeBC officer may be called to make a determination
- No worker is disciplined, dismissed, or subject to adverse employment action for exercising their right to refuse in good faith (WCA s. 150)

40.6 Penalty Provisions

WorkSafeBC has broad authority to issue orders, administer administrative penalties, and prosecute employers under the WCA. Key penalty provisions:

Violation Type	Potential Consequence
Failure to notify WorkSafeBC of serious incident	Administrative penalty up to \$595,441 (2024 scale); potential prosecution
Failure to comply with a WorkSafeBC order	Administrative penalty; prosecution
Willful or reckless disregard for worker safety	Prosecution; individual and corporate fines; imprisonment
Failure to cooperate with WorkSafeBC investigation	Administrative penalty; obstruction charges
Failure to maintain first aid requirements	Administrative penalty

Evergreen Community Care takes all WorkSafeBC orders and correspondence seriously and responds within the required timeframe. The Executive Director is notified immediately of any WorkSafeBC inspection, order, or investigation.

40.7 OHS Program Review Schedule

Review Trigger	Action Required
Annually (regardless of incidents)	Full program review by Program Manager and JHSC; document review completion and any amendments
After a serious incident or near-miss	Review relevant sections; update safe work procedures and controls
After a WorkSafeBC inspection with orders	Review ordered sections within required timeframe; document corrective actions
After a significant change in operations, staffing, or facility	Review impacted sections before changes take effect
After a change in legislation or WorkSafeBC policy	Update program to reflect new requirements; communicate changes to staff

APPENDIX K — FIRE HAZARD ASSESSMENT FORM

Facility Name: _____ Address: _____
 Date of Assessment: _____ Completed By: _____
 Program Manager Review Signature: _____ Date: _____

K.1 Ignition Sources

Ignition Source	Location in Facility	Condition / Control	Risk Level (L/M/H)
Gas stove / oven	Kitchen		
Electric stove / oven	Kitchen		
Portable electric heaters			
Furnace / HVAC	Utility room		
Water heater (gas or electric)	Utility room		
Electrical panel			
Microwave oven	Kitchen		
Dryer (lint accumulation)	Laundry		
Candles / open flame		Not permitted in facility	
Smoking / vaping		Designated area only; 3m from building	
Other:			

K.2 Fuel Sources and Storage

Fuel Source	Location	Quantity	Properly Stored? (Y/N)
Flammable liquids (cleaners, solvents)			
Combustible liquids (oils, cooking fats)			
Paper / cardboard accumulation			

Bedding / linen storage			
Oxygen (medical)			
Propane (BBQ)		Not permitted inside	
Other:			

K.3 Egress Assessment

Exit Point	Width (cm)	Obstructions Present? (Y/N)
Primary exit (main entrance)		
Secondary exit (rear / side)		
Basement exit (if applicable)		
Upper floor exit window (if applicable)		
Stairway exit		

K.4 Fire Safety Equipment Status

Equipment	Number / Location	Last Inspection Date
Smoke detectors		
CO detectors		
Fire extinguisher(s)		
Emergency lighting		
Exit signs		
Sprinkler system (if applicable)		
Fire alarm system (if applicable)		

K.5 Corrective Actions Required

Hazard Identified	Priority (1/2/3)	Action Required
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Next scheduled fire hazard assessment: _____

APPENDIX L — HEAT RESPONSE PLAN: ACTIVATION CHECKLIST

Facility Name: _____ Date: _____

Shift Lead: _____ Heat Alert Level: _____

L.1 Heat Alert Level Definitions (BC Heat Response Framework)

Alert Level	Temperature / Criteria	Organizational Response
Heat Warning	Forecast maximum 29°C+ for 2+ days (Metro Van); 35°C+ Interior	Activate Level 1 controls
Extreme Heat Emergency	Forecast maximum 39°C+ (Metro Van); 42°C+ Interior; or heat-related deaths in region	Activate Level 2 controls; consider relocation

L.2 Level 1 Activation Checklist

- Weather alert confirmed from Environment and Climate Change Canada or BC Weather Service
- Program Manager notified of heat alert activation
- All portable fans deployed to common areas and sleeping rooms
- Window coverings (blinds, curtains) on south and west windows closed during daytime hours
- Cool drinking water (chilled) available and accessible to all staff and youth at all times
- Community outings rescheduled to morning (before 10:00) or late afternoon (after 16:00)
- Staff briefed on heat illness recognition and response
- Youth briefed on heat safety and encouraged to stay cool and hydrated
- All staff and youth wellness checked at minimum every 2 hours during heat event

L.3 Level 2 Activation Checklist

- All Level 1 measures in effect
- Program Manager contacted to assess need for temporary relocation to an air-conditioned facility
- MCFD social worker(s) notified of heat emergency and any planned relocation
- Local cooling centre locations identified as alternatives for youth outings or daytime refuge
- Staff water breaks: minimum 250 ml every 20 minutes; breaks in cool areas mandatory
- After-hours emergency contact for youth's healthcare providers confirmed in case of heat illness
- Any youth or staff showing heat illness symptoms provided with immediate first aid and medical attention

Heat Response Plan Activation logged in shift log: Date / Time: _____

Program Manager Approval: _____ Time: _____

APPENDIX M — STAFF PSYCHOLOGICAL HEALTH REFERRAL GUIDE

This guide is for supervisors and staff seeking support for themselves or a colleague following a critical incident, traumatic event, or prolonged occupational stress.

M.1 EFAP (Employee and Family Assistance Program)

Access: Call the EFAP provider directly; no employer referral required. The toll-free number is on your Benefits Summary card and is also available from HR.

What is covered: Up to 8 free confidential counselling sessions per year per employee and eligible family members; financial counselling; legal consultation; nutritional counselling; life coaching.

Confidentiality: The employer receives only aggregate utilization data; individual use is strictly confidential.

M.2 WorkSafeBC Psychological Injury Claim

If you have experienced or witnessed a traumatic event at work and are experiencing symptoms of acute stress disorder or PTSD, you may be eligible to file a WorkSafeBC claim. This is covered under the Workers Compensation Act. To initiate a claim:

- Report the incident or cumulative exposure to your supervisor
- Complete the WorkSafeBC Worker Report of Injury or Occupational Disease (Form 6) at worksafebc.com or by calling 1-888-967-5377
- Your supervisor completes the Employer's Report within 3 business days
- Seek treatment from a registered psychologist or counsellor; keep all receipts and appointment records

M.3 Critical Incident Stress Debrief (CISD)

Following a Critical Incident (serious injury, near-fatality, unexpected death of a youth, severe physical assault), Evergreen Community Care arranges a professional CISD within 72 hours for all staff involved. The CISD is:

- Facilitated by a trained CISD or trauma-informed mental health professional
- Not a disciplinary or investigation process; focused solely on worker wellbeing
- Voluntary but strongly encouraged; no adverse employment consequences for not participating
- Followed up with individual EFAP referral for any staff who need additional support

M.4 Community Mental Health Crisis Resources

Resource	Contact
Crisis Centre BC (24/7)	1-800-784-2433 (1-800-SUICIDE)
BC Mental Health Support Line (24/7)	310-6789 (no area code required)
HealthLink BC (24/7 nurse line)	8-1-1

Vancouver Coastal Health Psychiatric Emergency	604-675-3900
FNHA First Nations Virtual Doctor of the Day	1-855-344-3800

APPENDIX N — SAFE WORK PROCEDURE: BLOOD AND BODY FLUID CLEANUP

Effective Date: _____ Reviewed By: _____

Applicable Legislation: WorkSafeBC OHS Reg. Part 6 (Biological Agents); WHMIS 2015

N.1 When This Procedure Applies

Use this procedure any time blood, vomit, urine, feces, saliva (with blood), or other body fluids must be cleaned up. This includes: youth medical episodes; injury first aid cleanup; bathroom incidents; and any other scenario involving body fluid on surfaces, floors, or equipment.

N.2 Required PPE

- Nitrile gloves (non-latex; minimum 8 mil thickness for cleanup tasks)
- Disposable apron or gown if large-volume spill
- Eye protection (safety glasses or face shield) if splash risk
- Closed-toe footwear

N.3 Procedure

Step	Action
1. Don PPE	Put on gloves before any contact with the spill area; add gown and eye protection if indicated
2. Contain the area	Place “Caution: Wet Floor” signs around the area; keep other youth and unauthorized persons away
3. Absorb the bulk material	Use paper towels or disposable absorbent pads to pick up the bulk of solid or liquid material; work from the outside of the spill toward the centre to avoid spreading
4. Dispose	Place all soiled absorbent material in a sealed plastic bag and place in the general waste (or biohazard waste bin if available)
5. Clean with detergent	Apply detergent solution to the area; scrub with a single-use cloth or mop head; remove detergent solution
6. Disinfect	Apply 1:10 bleach solution (1 part household bleach to 9 parts water, made fresh daily) or an approved disinfectant; allow contact time of minimum 1 minute; do not wipe off until contact time is achieved
7. Dry	Wipe dry with a clean single-use cloth or allow to air-dry
8. Remove PPE	Peel off gloves inside-out, touching only the inside surface; remove gown and place in waste; wash hands immediately with soap and water for 20 seconds
9. Document	Record the incident in the shift log; if an injury was involved, complete an Incident Report

N.4 Large-Volume Spills or Unknown Substances

For spills involving unknown substances, very large volumes, or suspected sharps in the spill area, stop and contact the supervisor before proceeding. Sharps in a spill area are removed using tongs or a sharps pickup device before cleanup begins.

N.5 Post-Cleanup

All mop heads, cloths, and cleaning equipment used for body fluid cleanup are laundered in hot water or disposed of. Mop buckets are rinsed and disinfected after each body fluid cleanup.

APPENDIX O — VIOLENCE INCIDENT DEBRIEF AND CORRECTIVE ACTION FORM

This form is completed within 48 hours of any incident involving physical aggression or significant verbal aggression toward staff. It is distinct from the Incident Report (Appendix B) and focuses on learning and prevention.

Incident Date: _____ Location: _____

Brief Description of Incident: _____

Staff Involved: _____ Supervisor: _____

Debrief Facilitated By: _____ Debrief Date: _____

O.1 Contributing Factors Assessment

Factor	Was This a Contributing Factor? (Y/N/Unsure)	Details
Youth trigger not identified or known		
BSP strategies not implemented		
Staff unfamiliar with youth's BSP		
Inadequate staffing ratio at time of incident		
Environmental factors (noise, crowding, temperature)		
Time of day / shift fatigue		
Staff communication during escalation		
Lack of de-escalation time or space		
Physical layout of the facility (no exit route)		
Equipment unavailable (duress alarm, radio)		
Recent change in youth's medication or care plan		

O.2 Staff Wellbeing Check

Physical injuries: _____

Psychological impact described by staff: _____

EFAP referral offered? Y / N Accepted? Y / N

Follow-up support planned: _____

O.3 Corrective Actions

Action Required	Responsible Person	Target Completion Date

O.4 Systemic Trends

Has this youth been involved in physical aggression incidents in the past 30 days? Y / N

Number of previous incidents this year involving physical aggression toward staff: _____

Is a BSP review required? Y / N Requested from: _____

Program Manager Signature: _____ Date: _____

APPENDIX P — ANNUAL OHS PROGRAM AUDIT CHECKLIST

Audit Date: _____ Auditor(s): _____

Facility / Program Audited: _____

Previous Audit Date: _____ Previous Audit Findings Resolved: Y / N / Partial

P.1 Program Documentation

Element	Status (Complete / Partial / Missing)	Action Required
OHS Program document is current (reviewed within 12 months)		
All sections reviewed and signed by Executive Director		
Current WorkSafeBC premium class and rate confirmed		
Employer registration with WorkSafeBC current		
“In Case of Injury” poster posted at each facility		

P.2 JHSC Compliance

Element	Status	Action Required
JHSC established with required membership		
Co-chairs (employer and worker) elected		
JHSC meetings held monthly (or quarterly if fewer than 20 workers)		
Meeting minutes prepared and posted within 1 week		
JHSC members completed WorkSafeBC training		
Annual JHSC report prepared and		

submitted to WorkSafeBC		
JHSC inspection records complete for all required visits		

P.3 Incident Reporting and Investigation

Element	Status	Action Required
All incidents reported and Incident Reports on file		
WorkSafeBC Form 7 filed for all compensable injuries within 3 days		
Serious incidents investigated with root cause analysis		
Corrective actions from investigations documented and tracked		
No serious incidents (s.172) unreported to WorkSafeBC		

P.4 Training Records

Training Requirement	All Staff Current? (Y/N/Partial)	Action Required
New worker OHS orientation		
WHMIS 2015		
First aid (current certificates on file)		
Workplace violence prevention		
Physical intervention (Non-Violent Crisis Intervention or equivalent)		
Emergency procedures (fire, lockdown)		

Hazard-specific training for all assigned tasks		
JHSC member WorkSafeBC training		

P.5 Safe Work Procedures

Safe Work Procedure	In Place and Current? (Y/N)	Action Required
Blood and body fluid cleanup (Appendix N)		
Medication management		
Working alone		
Physical intervention		
Ladder safety		
Lockout/tagout		
Sharps handling		
Vehicle and transportation safety		

P.6 Facilities and Equipment

Item	Compliant? (Y/N/NA)	Action Required
Fire extinguishers inspected annually		
Smoke and CO detectors tested monthly		
First aid kits fully stocked and inspected		
Emergency lighting tested monthly		
Fire drills conducted per schedule (quarterly)		
Workplace inspections conducted monthly		
Electrical hazards		

identified and addressed		
Vehicle inspections current		

P.7 Audit Summary and Sign-Off

Total elements audited: _____ Compliant: _____ Action Required: _____ Critical Gaps: _____

Summary of Critical Gaps:

Overall OHS Program Rating: ☐ Excellent ☐ Satisfactory ☐ Needs Improvement ☐ Non-Compliant

Program Manager Signature: _____ Date: _____

Executive Director Signature: _____ Date: _____

Next Annual Audit Date: _____